



PARLIAMENTARY DEBATES

OFFICIAL REPORT

CONTENTS

COMMUNICATION FROM THE PRESIDENT—

Absence from Ghana— [Col. 39]

VOTES AND PROCEEDINGS AND THE OFFICIAL REPORT— [Col. 2]

URGENT QUESTIONS— [Col. 34, 49]

ORAL ANSWERS TO QUESTIONS— [Col. 55]

STATEMENT—

Ghana Cedi at 60— [Col. 87]

PAPERS—

(a) Annual statement by the Audit Committee of the Pharmacy Council for the year 2024.
— [Col. 33]

(b) Annual statement by the Audit Committee of the Nursing and Midwifery Training College for the year 2024. — [Col. 34]

ADJOURNMENT— [Col. 112]

Correction of errors of substance may be made only on the floor of the House with the permission of the Speaker. However, correction of typographical or grammatical errors which Members suggest for the Bound Volumes which will be compiled at the end of the Meeting may be clearly marked in the Daily Report, and the copy containing the corrections submitted at the Editor's Office, Parliament House, not later than four clear days after the publication of the Daily Report.

BOUND VOLUMES of the Official Report (each with a comprehensive Index) are issued periodically during the Session. There is no fixed subscription rate, but prices will be quoted on each publication.

*Published by Office of Chamber Reporting,
Parliament House, Accra.*

*Designed and Printed by Publishing Department,
Parliament House, Accra.*

©2025 Parliament of Ghana.

THE
PARLIAMENT OF THE REPUBLIC
OF GHANA

THIRD MEETING, 2025

Tuesday, 28th October, 2025

*The House met at the Chamber,
Parliament House, at 2:24 p.m.*

[THE SPEAKER, RT HON ALBAN
SUMANA KINGSFORD BAGBIN,
IN THE CHAIR]

[PRAYERS]

The Speaker: Hon Members, before the roll call, I would want to acknowledge the presence of some visitors. We have 50 students from Prince of Peace Child Development Centre. They are led by the project director, Mr Enoch Asiamah Addo and they are from the Kwabre East Constituency. They can be seen on the right side of the Speaker at the public gallery.

I warmly welcome them to the House and encourage them to keep coming.

They may not learn much today because today was a bit heavy for all of us. Apart from the national shock we had as a result of the loss of our former First Lady, today, we had to celebrate the 60th Anniversary of the introduction of our currency, the Ghana Cedi. So, we do not have much today but you would get a feel of what the House is about. Some Hon Members would give you a taste of it so you may resume your seats and

watch what is happening so that in future, you can do it better than we are doing.

[Students of Prince of Peace Child Development Centre were acknowledged.]

The Speaker: Hon Members, let us now take the roll call. I call upon the Clerks-at-the-Table to call out your names and you know what to do because it is clearly stated in the Standing Orders.

[ROLL CALL]

The Speaker: Hon Members, we would now have to take the next item. We will have to recite the National Pledge. Please, may we be on our feet.

[THE NATIONAL PLEDGE]

**VOTES AND PROCEEDINGS
AND THE OFFICIAL REPORT**

2:54 p.m.

The Speaker: Hon Members, we would now take the item numbered 6 on the *Order Paper*, *Correction of Votes and Proceedings* and the *Official Report*. We would start with the *Correction of Votes and Proceedings* of Friday, 24th October, 2025.

Page 1...8—

Mr Boamah-Nyarko — *rose*—

The Speaker: Sorry, Hon Member?

Mr Isaac Boamah-Nyarko: Mr Speaker, first and foremost, I have seen

Hon Members complaining about the numbering of the pages. It is as if some of the pages are missing, but it is actually due to—So, from page 6, the next page is numbered 9, and page 7 comes after page 10. So, I just want to assist my Hon Colleagues that it is just the wrong placement of the pages.

Mr Speaker, most importantly, on page 6, I have inadvertently been captured absent even though I was in the House on Friday. So, I would want the Table to take note.

The Speaker: Well, your first point is not so with my copy. Maybe some copies have—all of you?

Mr Boamah-Nyarko: Maybe, it is with the Minority Side—*[Laughter]*—

The Speaker: Well, fortunately, I am neither Minority or Majority. I am using the copy available to me, and that one has not got the omissions you are talking about. But I have noticed that you have been captured as absent. So, I will take that one and draw the attention of the Table to the matter you just raised, that you were really present and not absent. So, Table, kindly take note of that.

Please make sure they get the right copies of the *Votes and Proceedings* of Friday, 24th October, 2024. All Hon Members should be given the right copies.

Hon Members, we go back to page 7...10—

Mr Frank Afriyie: Mr Speaker, on page 9, the item numbered 8(d), the very last line, a minor observation though, the full and proper designation of the Ministry is Ministry of Energy and Green Transition. So, the Minister could be appropriately captured.

The Speaker: Thank you, Hon Member, for drawing our attention.

So, we would go to page 10...11—

Mr Frank Afriyie: Mr Speaker, item numbered (c), line 5. With your guidance, the second word “extoled”, the past tense should be “extolled”.

The Speaker: Table, kindly take note of that.

Page 11...13?

Hon Members, in the absence of any further corrections, the *Votes and Proceedings* of the fourth Sitting, dated Friday, 24th October, 2024, as corrected, is adopted as a true record of proceedings.

I do not have any copy of the *Official Report*, so, we will move on to the next item on the *Order Paper*.

Yes, Hon Majority Leader?

Mr Mahama Ayariga: Mr Speaker, ordinarily, when there is a major occurrence, and this Parliament is bestowed with honours, we expect a Message from you through the normal Formal Communications by the Speaker. But when we got to the item numbered 5, I noticed you skipped it, and you did not tell us the great fortunes, the great happenings in Geneva, Switzerland,

where the Rt Hon Speaker was unanimously elected as the Chairperson of the 52-Member African countries, Africa Geopolitical Group of the Inter-Parliamentary Union. [*Hear! Hear!*]

The Inter-Parliamentary Union is the oldest International Parliamentary Association. It is the only global parliamentary association. We have the Commonwealth, which is just the Commonwealth countries, the Francophonie, which is just the Francophone countries, then we have the sub-regional and continental blocs, but one union has all the world's Parliaments and Legislatures being part of it, and that is the Inter-Parliamentary Union. As far as the 1950s, a bloc was formed out of that, which is the Africa Geopolitical Bloc, which is made up of the 52 African Parliaments.

Last week, they unanimously elected our Rt Hon Speaker as the Chairperson of that bloc, and I want to convey a message of congratulations of this House to the Rt Hon Speaker. Your continued election to lead major continental associations of Speakers is testimony to the fact that you have become a colossus when it comes to parliamentary diplomacy globally [*Hear! Hear!*]

You have come to represent what is good about parliamentary diplomacy, and not just that, but you have led this Parliament consistently for the past four years, and when it comes to national parliaments in Africa, we have been adjudged as the best most open parliament. [*Hear! Hear!*]

3:04 p.m.

Mr Speaker, we want to congratulate you. We take inspiration from the work that you have done. Among parliaments, and wherever we go when Ghana is mentioned, we have so much respect as a Parliament. Indeed, Mr Speaker, we know that due to the work that you have done, even as far as Canada, have requested for assistance from you to copy some of the innovations that you have introduced in this House. [*Hear! Hear!*] So, Mr Speaker, we wish you long life and good health, so that you can continue to offer this Parliament the needed guidance and help deepen parliamentary democracy and the independence of the Parliament of Ghana as a coordinate branch of Government.

I thought this should be brought to the attention of this Parliament. Upon your election, you brought great fortune to the Parliament of Ghana. Immediately you were elected, Ghana was given the seat of the Youth Parliament. So, Ghana now has one of our Members elected to also serve on the Youth Parliament of the Inter-Parliamentary Union (IPU). Mr Speaker, the seat was available, and Hon Comfort Doyoe Cudjoe — [*Uproar*]

Mr Speaker, upon your election the post became available for the selection of a young Member of Parliament. [*Uproar*] Hon Comfort Cudjoe gallantly fought and grabbed that position for Ghana. [*Hear! Hear!*] Afterwards, she relinquished it and transferred it to Hon Nasira Afrah Gyekye, who is in the Chamber now, upon your instruction.

Mr Speaker, we want to thank you very much for the opportunity to also occupy the youth forum position which has been offered to a female Member of this House.

With those few words of commendation and congratulation, I want to crave your indulgence to also draw your attention to a matter that came up after you left. After you left, the Human Sexual Rights and Family Values Bill (LGBTQ+ Bill) was advertised, and the following day, it was immediately taken out of the *Order Paper*. And I mentioned to this House that, truth be told, you had seen the adverts, but said there were few issues that needed to be resolved before a decision could be taken as to what to do. Because I took a position that, so far as I recollect, in the last Parliament, we passed the LGBTQ+ Bill, and my understanding was that it was transmitted to the Presidency. Later, we saw a communication from the Presidency threatening us to cease and desist in attempting to transmit the Bill to the then President for his assent. I did not know whether or not the Presidency was in receipt of the Bill, so I took a position that this House had passed the Bill, but we should wait until you return so that you can give us some direction as to how to treat this Bill.

I want to place on record that the Government of President John Dramani Mahama and the NDC are very ready to take this Bill. We are ready to assent to it, we are ready to take it in this House, but Mr Speaker, in view of the thorny legal issues that I raised, we were waiting

for you to come and give some direction so that we will know how to proceed. Whether we should have the Bill relayed and taken through all the various stages of consideration, or treated as having been passed and transmitted. So, we made that public, that is why I am raising it. It was discussed in this Chamber, that when you resume, we will draw your attention so that you give us some direction as to how to proceed.

Mr Speaker, those two are the issues that I wanted to bring to your attention and to get some direction from you.

Thank you, Mr Speaker.

Alhaji Habib Iddrisu: Thank you, Mr Speaker. In the Majority Leader's attempt to hijack the glory we chalked together in Geneva, which he was not there, he breached and offended our Rules. With the Statement the Majority Leader made, according to Order 93(1) of our Rules, we should be taking correction of *Votes and Proceedings* before we proceed. If he wants to make Statements of urgent public importance, he has to come under Order 93(1). The Majority Leader was supposed to go to Geneva, but he did not. It was the Speaker and me who fought for this position. The Majority Leader was not there but I was. [*Hear! Hear!*] We fought for this position. When we returned, we indicated to the Speaker our readiness to make an official Statement, but Mr Speaker said many of our Members were still in Geneva, so we should wait until everybody had returned before we could make that Statement.

Nevertheless, it is a call in the right direction and I also want to add my voice that, it was not an easy contest because many other countries had shown interest in the position of the Speakership of the African geopolitical group of the IPU. Senegal and Nigeria were very forceful on that, but upon reading the experience and the background of our Speaker, those countries withdrew. Having said that, our Speaker was a Member of Parliament from 1993, became a Deputy Speaker of the House, a Minority Leader for eight years, a Majority Leader for five years, a Minister for Health, and at a point, part of the three wise men. Having been a member of the IPU since 2001 when he was a Deputy Speaker, Nigeria withdrew, indicating that even though they had interest and were ready for election, our Speaker was experienced and dedicated, having been a member of IPU for close to 24 years. Senegal had made a serious case of IPU, making reference to their President being around 45 years, their Prime Minister around 50 years, and their representative also being a young man, but because of the experience of the Rt Hon Speaker, they also had to withdraw.

3:14 p.m.

Mr Speaker, on behalf of our Colleagues in the Minority—Mr Speaker, the Hon Majority Leader was not part of that success story; my Hon Colleagues and I were part of that success story. We fought for and were present to ensure that this success story happened.

If you asked Ms Cudjoe, when the position was made vacant, we said we

should propose the Rt Hon Speaker. And, Mr Speaker, I was happy that at that point, we were not partisan. We were also not partisan. When we got to the turn of the youth parliamentarian, Ms Cudjoe said for the youth parliamentarian, it had to be Alhaji Habib Iddrisu. But when they made the indication that it had to be a woman, as young as Ms Cudjoe — *[Laughter]*. Mr Speaker, she is as young as 63 years, and for that matter, that position was given to Ms Cudjoe. I am happy that upon further deliberation, she ceded the position to a much younger person who will be part of the IPU. It is good news because our First Deputy Speaker, Mr Bernard Ahiafor, has also been an executive member of the IPU, and the Rt Hon Speaker is now the Chairman of the IPU. We also have one of us with the young parliamentarians. Mr Speaker, we congratulate you. We are happy and we know you are experienced and that you will be able to make us proud.

Mr Speaker, the second leg of my submission has to do with what the Hon Majority Leader spoke about, the Human Sexual Rights and Ghanaian Family Values Bill. The Hon Majority Leader is a Harvard trained lawyer, but sometimes he leaves his legal thinking at home, and would not want to—*[Uproar]* Mr Speaker, the issue we raised on the Floor was that, on the day we resumed, the Human Sexual Rights and Ghanaian Family Values Bill was on the *Order Paper* but it was missing from the *Order Paper* for the next day. So, I drew the attention of the Hon Majority Leader to it.

Mr Dafeamekpor — *rose* —

Alhaji Habib Iddrisu: If my Colleague is on his feet because of the earlier statement I made about the Hon Majority Leader, it was not intended to offend him, but if it does, I retract it. He knows I respect him. When I said he is a Harvard trained lawyer, I was just building my point.

Mr Speaker, the argument the Hon Majority Leader was advancing, that he thinks that the Bill is passed and asking why the President did not sign, is very surprising. If it has been passed, he is the Majority Leader and Leader of Government Business, so why is it that for 10 months, the President has not signed it? Mr Speaker, we all know that when the Eighth Parliament died, everything died with it. Even if you say that we should leave it as it is and that the President might make comments on and return it to Parliament to consider some sections of it, you should be mindful that some of the sponsors of the Bill who were members of the Eighth Parliament are no longer in the Ninth Parliament. So, Hon Majority Leader, it cannot be the case that it was passed. It cannot be. The right thing to do is to reintroduce it and then we go through the process. But if it is the case that the Majority Side now accept men holding hands and kissing at the Independence Square and they do not care, then that should not be the case—

The Hon Majority Leader indicated to us that when you return, you would be able to address the issue. We are happy that he raised the issue himself. We would wait for directives from the Rt

Hon Speaker to make sure that this issue is laid to rest. If the Hon Majority Leader is offended by the earlier statement I made, I withdraw it and apologise.

Thank you.

The Speaker: Hon Majority Leader?

Mr Ayariga: Thank you, Mr Speaker.

Mr Speaker, yes, I was not in Geneva because Parliament was scheduled to resume on the 21st October, which was Tuesday, and I decided to prioritise the resumption of Parliament and then subsequently join the team in Geneva. The weekend to the resumption of Parliament, the Assembly had already elected the Rt Hon Speaker as Chairman of the IPU. So, when I called the Rt Hon Speaker and said I wanted to come immediately after we resumed, he said the election had taken place. I said, well, if the election has taken place, I would not want to waste Parliament's money by flying to Geneva. So, I do not see how not being there when all of you were there—

I can assure you that, when the Hon Member says the election was not easy, that is not true. He was there. The election was so easy because of the candidate we put up. [*Hear! Hear!*] He was there, but could not report exactly what happened. I was not there, but I know that the Rt Hon Speaker did not even put himself up; he did not. He was there, he knows. The Speaker did not. I got a very detailed report from the Rt Hon Speaker—[*Laughter*—]—and all those who were there. Senegal, Nigeria

and Sierra Leone were interested; they were all over the place, but everybody was looking at the Rt Hon Speaker to see if he wanted it. Immediately, the Rt Hon Speaker gave an indication that he would not mind taking the candidate, everybody withdrew. Is that not a very easy election? *[Laughter]* So to say that the election was not easy when the Hon Member was there is a total misrepresentation of what happened. He should report accurately. The election was so easy because it was the Rt Hon Speaker.

Secondly, on the issue the Human Sexual Rights and Ghanaian Family Values Bill, I remain convinced that the fastest way to enact the law is if we assume that the Bill has been passed by Parliament, in which case, what is left is just the assent; that is the fastest way. If he will recall, the Bill spent close to a year in this Chamber. So, if we bring it back and start all over again, I cannot tell how long the Bill will be in this House, alright?

Now, I took a position that once the Bill was transmitted to the Presidency, it was within the bosom of the Presidency to decide what to do with it. But after Sitting, I consulted the Drafting Office, and the Drafting Office reported to me that the Bill was never transmitted to the Presidency.

That was the report I got, that because officials appointed by the New Patriotic Party (NPP) sitting at the Presidency wrote a letter threatening this Parliament not to bring the Bill there, the

party of the Hon First Deputy Minority Whip threatened Parliament not to bring the Bill to the Presidency. *[Uproar]* And today, he is sitting here and pretending he is so interested in having the Bill enacted.

3:24 p.m.

When they had every opportunity to have the Bill enacted and assented to by President Nana Akufo-Addo, they refused to give the assent. There were demonstrations in this country to get the assent, and they refused. Some of us went to court to try to get a *mandamus* to compel him, but they refused. Today, they are pretending to Ghanaians that they are so interested in the LGBTQ legislation. They are creating an impression that we, in the NDC, who did everything to get them to give the assent, are no longer committed to the Bill.

I thought the fastest way was to treat the Bill as having been passed. But if they want us to start all over again, I want to assure them that this Side of the House is ready. So, Mr Speaker, like I indicated, we are in your hands. Give us directives on what you think. I am told that the Bill was never transmitted to the Presidency. Mr Speaker, if the Bill was not transmitted to the Presidency to be assented to after it had been passed by this House, it creates quite a legal issue. So, I thought that we should wait for you to come and give us some direction on the matter, then we can proceed.

But let me give every assurance that we, on this Side, are ready to pass this Bill as a House. And let me also assure

them that I have personally spoken with the President and he has no hesitation to sign, if this Parliament should pass the Bill. Unlike his predecessor, who did everything to avoid giving assent to this Bill, including threatening Parliament.

I am surprised that a sitting President threatened the Legislature, and nothing has been done about it. A secretary to the President wrote a letter threatening the Clerk to cease and desist every effort to present a Bill duly passed by this House, and nothing has happened to him. This raises serious constitutional matters, and if the autonomy and integrity of this Parliament are to be restored, action must be taken. He should be referred to the Committee of Privileges and Immunities for writing to and threatening this House from exercising its constitutional responsibility. Mr Speaker, this is a matter that should go to the Committee of Privileges and Immunities.

So, we should not countenance a behaviour like that where this Parliament is stopped from performing its constitutional duties. But Mr Speaker, be that as it may, as I have indicated, we are in your hands, waiting for your direction. Tell us what you think, and we will do as you direct.

Thank you, Mr Speaker.

The Speaker: Hon Members, let me acknowledge the presence of some of our students from the Akuapem South Constituency. We have students from Berekuso M/A Junior High School. They are 65 in number and are led by the head teacher by name Mr Richard Adom. You

can see them on the left side of the Speaker at the public gallery.

I warmly welcome them to the House, and please urge them to take their time and listen to what is happening here. What we are doing here is to try to catch them young and build a parliamentary democratic culture in the country — They are at the other side —

Hon Member, I hope and pray that you stop interrupting the Speaker. Read your Standing Orders. You listen to the Speaker in silence. It is clear there. You are almost one year old now, and you should have learnt a lot. This time, I will not temper justice with mercy. [*Hear! Hear!*]

So you are warmly welcome, and please, you may resume your seats.

[Pupils of Berekuso MA Junior High School were acknowledged.]

The Speaker: Today, I allowed this conversation to go on because we have our brothers and sisters here from the Parliament of the Republic of Uganda. Some people call it /u: 'gændə/. They are on a study visit from Monday to Friday, the whole week. They will be with us here, and as usual, they are at the Distinguished Visitors' Gallery. They are being led—in fact, they are a 13-member delegation led by the Leader of the Opposition, Hon Joel Ssenyonyi. Sorry if I did not get the pronunciation well. Is it right?

Yes, I was in your position in 2001 as leader of the opposition, and so it is good that you are here with your Members. He

is accompanied by five other Hon Members of the opposition and seven officers of the Parliament of Uganda. This is a benchmarking visit, and so I allowed this exchange to go on as part of the benchmarking. I have been to their Parliament many times, and their current Speaker is a beautiful lady, Rt Hon Anita Annet Among. I think in her constituency, she is unopposed. They are having elections now, and she is unopposed in her constituency.

But the politics in Uganda, even though we are all together in the Commonwealth, has a different type of intensity than ours. And so it is good to have this benchmarking. We have sent our Members there, including our staff, from time to time, and I am sure you know each other. So please get some time, share your ideas and experiences with them and learn from them. They have a well-composed and well-structured opposition in that Parliament, in spite of the paucity of strength. But they may also call themselves the “mighty minority”.

Please, share these practical tools and some of the approaches that we are adopting here. You mentioned in your speeches some of the things that we have done and how we have succeeded in completing a tenure of a hung Parliament in Ghana, which is not common at all in the world. Wherever you have a hung Parliament, they fail to complete it. Even in Ghana, when we had one, it did not end its tenure. That was what we had in the 1979 to 1981 Parliament. It was also a hung Parliament.

Now the Leader of the Opposition is accompanied by, as I told you, five other Members. One is Hon Hellen Nakimull.

3:34 p.m.

Yes, that is one of the young ladies here. I will be talking about the youthfulness of Ms Comfort Doyoe soon—*[Laughter]*

Then Hon Flavia Kalule. We also have Hon Kawalya Abubaker and Hon Lüyimbazi Nalukoola. The fifth person is Hon Mpalanyi John Paul. For the staff, we have the Director, Ms Ruth Ekirapa Byoona and Mr All Mivule who is the Public Affairs Officer. We also have Principal Policy Analyst, Mr Ssozi Leonard and the Principal Administrative Assistant, Ms Timugiibwa Irene. We have an Administrative Assistant in the person of Ms Naluyima Phoebe and a Policy Analyst Mr Lusiba Ali Luwukya. The last but not the least is an Assistant Secretary, Mr Patrick Okello.

[The delegation of Minority MPs and Staff from the Ugandan Parliament were acknowledged.]

Hon Members, on your behalf, I want to welcome them to the Parliament of Ghana. This is the 9th Parliament of the Fourth Republic of Ghana. We have been along this since January 1993. You are most welcome. I wish you fruitful deliberations, and please take time off to also experience life outside the precinct of Parliament.

Hon Member, as I stated earlier, they are to be with us for one week which started yesterday. I am reliably informed that they were here yesterday, and they were in the bosom of the Hon Minority Leader, Osahen Afenyo-Markin, for the whole of yesterday—Well, that is the

proper thing to do. They are from the opposition, so the best ally is the leader of the opposition. I have gone through it before, so I know. I want to hear what he has to say about his first impression and what he has shared with them. I will give him five minutes.

Minority Leader (Osahen Alexander Kwamena Afenyo-Markin): Mr Speaker, thank you for the opportunity to say a few words. Yes, it is true, our friends from Uganda were my guests in Winneba yesterday. We spent a good time to share views on democracy. Mr Speaker, we discussed you a lot. I told them that I have spent a whole lot of time with the *Hansards* of your era, and I draw a lot of inspiration from that period of turbulence in your party—*[Laughter]*—You steered the affairs of the party in this very Chamber, facing the mighty men and women from the likes of Rt Hon Peter Ala Adjetey.

Mr Speaker, they were concerned about the 40 years of one-party state where it appears they cannot have a change in government. I told them, well, Ghana started quite rough, and we endured 11 years of dictatorship, military regime and even the first eight years where we did not have transparent ballot box. We were using thumbprint Identity Cards (ID) cards. Through the industry of the minority, we managed to transform our system, and today we have it. We all understand as political actors that if you do it well, you will have the benefit of government. So, in Ghana today, we do not need to fight. We need to rely on the rules to show relevance. It does not matter the huge numbers, I told them. *[Laughter]*

Mr Speaker, they say they have elections come January, 2026. I wished them well and told the young barrister who is leading the minority, he is 38 years, that they need to work hard and use all the tools available. I have shared some important data with them and how we have held our own. I told them it is about the collective spirit and not the competition among themselves. The competition would only divide them, I told them. I also told them how difficult it is to be a Minority Leader than to be a Majority Leader. Mr Speaker, you and I know. Hon Ayariga is not part of our club of Minority Leaders. We know what it is. He is enjoying Majority leadership. It is easy to be a Majority Leader, but it is difficult to lead the Minority, especially when you are few.

Without more, I would want to say thank you for the platform you gave to the group from Uganda. We shared ideas. All we need is our democracy. We are here in Ghana and we do not seem to value what we have. But if you look at the continent, we have really advanced in our efforts. What I think we should do is to strengthen the institutions, and make the institutions formidable. I am sure if we do that, Ghana will be far better a place for others to come and learn from. Thank you, Mr Speaker.

The Speaker: Yes, Hon Members, it is a proper thing to do, to listen to the Minority Leader. As I have been saying, democracy is about opposition. That is what democracy is about. It is good that you are here from Uganda as brothers and sisters because the current President of Uganda actually studied in Ghana, and

he was at Winneba. The current Minority Leader in Ghana is representing the good people of Winneba, Effutu. That time, as a young man, he studied at the Kwame Nkrumah Ideological Institute at Winneba, and he became a member of the Convention People's Party (CPP) before he left Ghana. So, he has risen through that, and he has a lot of organisational ability. He has written a lot of books, and I have had the honour of interacting with him a number of times.

3:44 p.m.

Anytime I am in Uganda, I try to meet both sides and have discussions. So, you are most welcome. It is an unending journey, and sometimes very rough, but that is part of the game. Even those we see as mature democracies also still have rough times, as you can see happening in the United States of America (USA). So, please, do not hold back anything; share what you have with them. We are not an island; we are a continent, so what happens in Ghana affects Uganda, and vice versa, so we are working together.

But let it be known that the Hon Majority Leader has been a pillar since 2005, when he entered Parliament. He was a Ranking Member to the then Committee on Constitutional, Legal, and Parliamentary Affairs. I have known him since he was a student at the Ghana School of Law, so he has a lot of experience in parliamentary practice. Hon Ayariga, do you want to add more?

Mr Ayariga: Mr Speaker, my friends from Uganda are in the

opposition and they want to come to power. So, when they come to Ghana, take them to those who have successfully fought for power. *[Hear! Hear!]* You should refer them to those who have successfully fought and attained power so that we can teach them how to do it, not those who have lost power. For what is a man who has lost power going to teach them? We will teach them how to win power. They are welcome.

The Speaker: I was invited to Nigeria as a resource person when the then ruling party lost power, and they wanted to learn from me on how to live in opposition. That was the question they put to me; how to live in opposition? I reminded them that in 2003, when I attended a conference there, I introduced myself as the opposition leader. After the conference, a number of them came to me and said, "so are you a member of Rawlings' party?" I said "yes". Then they said, "so you people stood there and the opposition took power away from you?" I said, "we did all we could. But you know, one thing that is permanent in life is change." They said, "What? So, you sat there and they took it from you? It will never happen in Nigeria." I said, "we did not sit, we were standing but we lost." *[Laughter]* So, when they lost their power and invited me to come and share how to live in opposition, I shared with them.

In fact, I used to move with the Speaker and the Majority Leader as a team representing Ghana, so they asked me whether I was really opposing them, and I told them "yes". They said, "how can you be in bed with them and say you

are opposing them?” So, when we won the election in 2008, I was there. They said, “ah! so you were really opposing them?” To be able to oppose somebody very well, you must know the person. So, I know the NPP very well. I studied it from the time it was in the mother's womb till date. It is good that they started from the Minority. But they will definitely get in touch with you, so that they can learn from you on how you managed to transit from opposition and are now in Government with a huge majority in the House. As one presidential candidate used to say, “it is possible”.

Hon Members, I want to thank the House for appreciating the achievement of the delegation at the IPU. Our Parliament is highly respected. It is not just the individual, but the Parliament of Ghana. We have made a lot of headlines, achieved a lot, and will continue to work together to achieve more. The delegation was made up of both Sides of the House; we worked together and got the positions that they mentioned.

Hon Comfort Doyoe Cudjoe is actually a young Member of Parliament (MP) because when I met her, she was a women's organiser, and I encouraged her to contest to come to Parliament. I was able to inspire her. She contested and since she entered Parliament, nobody has been able to beat her in her Constituency. *[Hear! Hear!]* So, to me, she is really a young MP even though she is now a fourth-timer, going to 16 years. So, what she did was to present herself. A number of them looking at her, doubted her age, so they shouted, “you are the young

MP?” She said, “yes, she is the young MP”. But she managed to hold on to the position. We had to cross-check because the rules say that the MP should be less than 40 years in age, not parliamentary experience. That disqualified a lot of our women because a lot of them are above 40 years, but we had a few that are 40 years.

I used the usual fairness in deciding, which was accepted by the delegation, that we should give it to the young lady because the others are in other delegations. That is how come I zoned on the Hon Lady from Sekyere Afram Plains, Mrs Nasira Afrah Gyekye. She is the one going to represent us at the Forum of Young Parliamentarians. The Forum of Young Parliamentarians had opted for another person, but I drew their attention to the fact that that person is already in another parliamentary delegation. So, it is fair to share and not have one person with two or three delegations while others have not. That is the fairness I applied. Sometimes some people are not happy about it, but so far as I am alive, that is what will continue.

Hon Members, with regard to the Bill, as you know, the Eighth Parliament is history, so are all the Businesses that were pending in the Eighth Parliament. We have a new Parliament, the Ninth Parliament, therefore what was pending there came to an end and had to be reintroduced. In spite of the challenges I had with the former President—It was actually transmitted to the former President and they wrote to tell us that they will not assent to it. The only constitutional error they committed was

that they did not give us the reasons they did not assent to it. They did not also transmit the Bill back to us, but they imprisoned it there, so we did not get the Bill back.

3:54 p.m.

So, the Ninth Parliament on resumption took back the Bill, and as you would recall, that is a Private Members' Bill, so Members of this House took it up; they have gone through the whole process, as detailed out in our Standing Orders. But as it went through the process, the new Government indicated that they wanted to take it as a Public Bill. I personally disagreed. I made it known to His Excellency the President that this is an initiative of our people; the traditional leaders, the religious leaders, and civil society leaders. Ghanaians wanted this Bill passed through Private Members' Bill, and so be it. We will process it as Private Members' Bill.

Now, the processes have been completed, and as you know, it has to come back to the Speaker. You know our Rules have given what the Speaker is expected to do, which I will do, before it can be laid in the House. That has not been done. So, when it was put on the *Order Paper*, I queried it, because you have not heard from me, according to the Standing Orders. So that will be done before it is laid before the House. But because a lot of work has already been done, we think that it is going to have a smooth process in the House, because both Sides of the House now support it, and His Excellency the President has

given indication that he is ready and willing to assent to it at any time that it is passed by the House.

Hon Members, this language was proposed by me, as the Chairman of the Constitutional and Legal Affairs Committee. Initially, our laws were not passed that way. But the 1992 Constitution is clear in its rendition. We had to debate this as to how to couch the introduction, or what we call the preamble of the Bills. The preamble of the Bills is legally termed the enacting formula of Bills. There was a debate chaired by the then Speaker, D.F. Annan, and I had to propose this. Everybody was convinced, and it has been adopted since then. That is why we have it as passed by the Parliament of Ghana and assented to by the President. That was not the enacting formula, and I think it has worked for us.

So please, hold your peace. It will come sooner than later. Civil Society Organisations (CSOs) are getting restless again, but I would want to assure them that they know my passion in this matter. I would want to get it properly passed, so that its implementation becomes easier. There are a few things that our attention has been drawn to, which we will work on together here, so that when we pass the Bill, it will be a Bill that is accepted by all, and we will definitely obey the prescriptions of the Bill. So please, I thank you all for your contributions.

I did not want to be the one talking about my being elected as the Chairperson of the 52-member Africa geopolitical group of the Inter-

Parliamentary Union (IPU). Please, brothers and sisters from Uganda, your Speaker of Parliament too is my vice in the Conference of Speakers and Presidents of African Legislatures (CoSPAL). I am sure she would have told you that. So, learn from both Sides; do not learn from only one Side. You could shift to Minority; you could also shift to Majority; opposition or government. Nothing is impossible. What God cannot do does not exist.

So, we proceed from here.

Yes?

Osahen Afenyo-Markin: Mr Speaker, thank you for always bringing to light important matters, but just by way of clarity, you indicated that there are some problems that have been identified with the Bill that was passed by the Eighth Parliament. In other words, the attention of the Rt Hon Speaker has been drawn to some few things.

Mr Speaker, we will be happy that at the appropriate time, some of these issues are shared with us. Because for us, this is a very important matter and I am happy that you have brought it to a peaceful rest considering the unyielding posture our Friends opposite me took on this matter. Even where we were proposing some amendments for safe landing and we had had discussions, they never supported it. And in a hot afternoon, they decided to hang us with that Bill and they were partisanly political with it. So, if today, as Mr Speaker has put it, they themselves have realised that the Bill they passed was

problematic and they would want to— **[An Hon Member:** The Bill was not problematic.] If today they have seen that their attention has been drawn to some excesses in the Bill, however too late in the day, it is good that they themselves have come to that realisation. But Mr Speaker, we wait patiently for that day.

Mr Speaker, our Friends must know that sometimes when they place a rat poison on another man's tongue, it does not take too long for they themselves to taste that rat poison. We are patiently waiting for this Human Sexual Rights and Family Values Bill, 2025. We are patiently waiting for the corrected version.

The Speaker: Hon Minority Leader, do not read a different meaning into what I said; just wait. You know Mr Speaker is very open, so whatever it is, I will let you know. But there is no corrected version. It may be an improvement, but that does not mean a corrected version. I think enough is enough —

Osahen Afenyo-Markin: Mr Speaker, I would have preferred their old version —

The Speaker: Resume your seat.

Yes, Hon Member?

Alhaji Suhuyini Sayibu Alhassan: Mr Speaker, thank you very much for the opportunity to point out a few inaccuracies in the presentation that was made by the Hon Minority Leader. Mr Speaker, you have always led and it has already been celebrated by the Majority Leader and other Colleagues on the floor with maturity. Your views and your

guidance have never been construed or misconstrued to be the views of any group in this House.

4:04 p.m.

Mr Speaker, I am therefore surprised that, as you guided us on this matter, the Hon Minority Leader made it seem as if you were speaking the position of the Majority in the House and those who sponsored the earlier Bill.

Mr Speaker, I was part of those who sponsored that Bill, and as far as we are concerned, we passed that Bill in the state and time that we did, guided by good conscience, and we were hopeful that the President at the time would have signed it.

Mr Speaker, I would not go over the issues that happened then, but if it has become necessary for this House to reconsider it, I take solace in the assurance of the Majority Leader that we stand in readiness to support the process of enacting a law that will make the practice intolerable and unacceptable in this House. We are not shirking from that responsibility or moving from that position that we took earlier. If they have changed their position, I would have wished that they would have done so with their former President. But even if they are unable to, we welcome them on board, because this is a cause for Ghana.

Thank you, Mr Speaker. [*Hear! Hear!*].

The Speaker: Hon Members, I am happy that our brothers from Uganda are

here, because they are ahead of us already on this matter and so they will share their experiences with us. They have already passed an Act on it and implementing it. Having hosted two conferences on this subject matter, they are calling on other countries, particularly Ghana, to host the next conference. So, they would share their experiences with you and maybe convince us to host it for other African countries to emulate the good examples that we are setting.

Yes, Hon Samuel Abdulai Jinapor?

Mr Samuel Abdulai Jinapor: Thank you very much, Mr Speaker. I just want to respectfully observe that I listened to you very carefully when you were giving the guidance because the matter that was dealt with in this House in your absence, which the Majority Leader invited you to clarify, was a matter of great concern to me.

Mr Speaker, I was very eager to pick your direction and guidance on the matter. You resolved two matters, as I understood it.

Mr Speaker, first, you resolved the matter of whether the Bill is pending, which therefore can be assented to by the President. You guided and gave the direction that the Eighth Parliament dissolved with all its uncompleted Acts. Therefore, the Bill which was passed in this House died with the Eighth Parliament and it no longer exists. Mr Speaker, quite clearly, that position is in consonance with the position taken by the Minority Leader and this Side of the

House. With the greatest respect to the Majority Leader, I am sure he will concede that the position he articulated or submitted in respect of that particular matter is at variance with the guidance the Speaker rightly gave.

Mr Speaker, the second one is what Hon Suhuyini spoke to.

Mr Speaker, you said there have been certain proposals to make the Bill better, and which is what the Minority Leader pointed out. That if you have realised that the Bill that was passed by the Eighth Parliament was not fit for purpose, and therefore you have found it necessary — Mr Speaker did not give specifics as to what the specific proposals or variations to make the Bill better are but the point the Minority Leader made is that there seems to be a certain concession that the Bill that was passed by the Eighth Parliament was not fit for purpose. Therefore, it has been revised for this Ninth Parliament to consider it.

Mr Speaker, it would be very interesting for us to find out what the specific areas which have been revised are. This is because for example, one of the contentious issues which came up in respect of that particular Bill was the issue of human rights.

Let me conclude —

The Speaker: Hon Member, we are not debating the Bill. Now you are going into contents. You are not referring to what I said or what he said but you are now going into contents. We are not

discussing or debating any provision of the Bill and you still do not know what I meant by the statement. So please, I assure you that when we get there, being an open Speaker, I would let you know what it is.

Mr S. A. Jinapor: Mr Speaker, I am guided but let me just conclude by saying that the Bill should be brought as it was and as was passed by the Eighth Parliament in the same form and shape — [*Uproar*]— they should not change or water it down. What Ghanaians want is to have a law which promotes good sexual conduct in Ghana and prevents LGBTQ. They should not change the Bill from the Eighth Parliament, water it down, revise it and bring it to the Ninth Parliament and have the President assent to a Bill which is not fit for purpose. We want the Eighth Parliament Bill for the Ninth Parliament to consider. It should be verbatim and not changed.

That is the point. Thank you, Mr Speaker.

The Speaker: Hon Member, what we are talking about is not a Bill coming from the Majority side. What we are talking about is a Private Member's Bill co-sponsored by both Sides of the House. Even in the Eighth Parliament, it was co-sponsored by both Sides of the House. It was not from one Side. It is still the same co-sponsorship of both Sides of the House. You have not seen the contents, and that is why I said you should hold your peace. When we get there, you would get to know what I referred to. I know that this is the highest political or partisan platform of any

democratic country and so you can keep on chipping at it with your political game, but the truth will remain the truth. Nothing can happen to it. Please wait until I inform you about the situation. Then we can take it on and debate.

Hon Majority Leader, may we move on to Urgent Questions?

Mr Ayariga: Mr Speaker, I do agree, but I was hoping that the same Minister could lay his Papers before answering the Questions.

Mr Speaker, if you would indulge me so we could vary the order of Business to take the item numbered 10, and then we now take the Questions. The Paper is to be laid by the same Minister. He would just bow and then start answering the Questions.

The Speaker: Yes, Minority Side?

Alhaji Habib Iddrisu: Mr Speaker, we do not have any difficulty with that.

The Speaker: Well, Hon Members, in accordance with our Standing Orders, vary the order of Business for today and move straight to the item numbered 10, at the commencement of Public Business — Presentation of Papers.

PAPERS

By the Minister for Health (Mr Kwabena Mintah Akandoh) —

- i. Annual statement by the Audit Committee of the Pharmacy Council for the year 2024.

- ii. Annual statement by the Audit Committee of the Nursing and Midwifery Training College for the year 2024.

Referred to the Committee on Health.

The Speaker: Hon Members, let us now go back to the item numbered 7 — Urgent Questions. We have two Urgent Questions standing in the names of the Members of Parliament for Afigya Sekyere East and Lambussie.

So, we start with the one standing in the name of the Hon Member for Afigya Sekyere East, the item numbered 7(a).

Hon Member for Afigya Sekyere East —

4:14 p.m.

Ms Mavis Nkansah-Boadu: Mr Speaker, I am here, on the Minority Side.

URGENT QUESTIONS

MINISTRY OF HEALTH

Challenges in Accessing Hepatitis B Immunoglobulin (HBIG) for New Born Babies

Ms Mavis Nkansah-Boadu (NPP — Afigya Sekyere East): Mr Speaker, I beg to ask the Minister for Health the challenges in accessing Hepatitis B Immunoglobulin for newborn babies.

The Speaker: Yes, Hon Minister?

Minister for Health (Mr Kwabena Mintah Akandoh) (MP): Mr Speaker, Hepatitis B immunoglobulin (HBIG) is a critical life-saving intervention administered to newborns within the first 12 hours of birth, to prevent mother-to-child transmission of the Hepatitis B virus. In cases where the mother is known to be Hepatitis B positive and has a high risk of passing the infection to the infant, the immunoglobulin provides immediate passive immunity while the newborn also receives the Hepatitis B vaccine series, including the birth dose. Combining HBIG with a birth dose of the Hepatitis B vaccine gives the best protection for high-risk infants.

Despite its importance, there are significant gaps in access to HBIG for newborns as outlined below:

The first issue is the non-inclusion in the National Health Insurance Authority's (NHIA) reimbursement list. HBIG is currently not listed among the reimbursable medicines under the National Health Insurance Scheme (NHIS), limiting its accessibility at a point of care.

The second issue is its limited availability. The product is typically sourced from the open market. While a few regional hospitals maintain limited stocks, most district-level facilities do not stock HBIG at all.

The third issue is the scarcity of vendors. On the open market, only a limited number of vendors supply HBIG, further constraining its availability. Information from the Food and Drug

Authority (FDA) suggests that there are only two products currently under registration.

The fourth issue is its high-cost and payment terms. HBIG is relatively expensive and is sold strictly on a cash-and-carry basis thereby exacerbating access challenges for low-income families.

The last issue is with the public health programmes, that is the routine immunisation package. On a positive note, the expanded programme on immunisation introduced Hepatitis B vaccine as a component of the five-in-one or pentavalent vaccine in 2002 and it is expected to introduce birth-dose of the Hepatitis B vaccine in due course with a routine Hepatitis B vaccination schedule continuing from six, 10 and 14 weeks of age.

I thank you, Mr Speaker.

The Speaker: Hon Member, any supplementary questions?

Ms Nkansah-Boadu: Mr Speaker, I asked this Question because on my routine visit to my constituency, I had a conversation with one of the medical doctors in my constituency and he informed me that majority of the mothers who happen to have Hepatitis B go home without giving their kids this vaccination because they cannot afford it. The frightening aspect about this is that he mentioned that the lifespan for such a child is 25 years. I want to find out from the Hon Minister if that is the case and what the Ministry is doing about it.

The Speaker: Yes, Hon Minister?

Mr Akandoh: Mr Speaker, as I indicated in my response, I agree that it is very expensive in the market, but I can assure you that government is going to include it in the routine or traditional immunisation very soon. Maybe, in the first quarter of next year.

The Speaker: Yes, anymore?

Ms Nkansah-Boadu: Yes. Mr Speaker, I do know if the Hon Minister understands how it is administered. It is supposed to be given within the first 24 hours of birth. But when he says routine— then that means the child will be a carrier. So, when he says routine, it means a few months after that, but within 24 hours, if the child is not given that vaccine, the child will carry that disease for the rest of their lives.

The Speaker: Yes, any clarification, Hon Minister?

Mr Akandoh: Mr Speaker, what is not part of the routine immunisation is not part as it stands now. It is not, and what I am promising the Hon Member is that we are rolling that aspect onto the routine immunisation. So, when we start, from that point, it would be part of the routine immunisation. I hope the Hon Member is clear now.

Ms Nkansah-Boadu: Mr Speaker, I think —

The Speaker: Please, let her finish her last question.

Ms Nkansah-Boadu: Mr Speaker, the Hon Minister is still not getting me, but that is fine. For my last question, I want to know when the Hon Minister plans on rolling that out.

The Speaker: I thought he answered that. He did.

Yes, Hon Member?

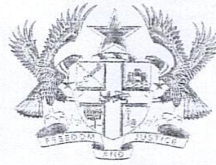
Dr Abdul Kabiru Tiah Mahama (NPP — Walewale): Mr Speaker, the Answer of the Hon Minister to the Question raised by the Hon Member suggests that the cost of this immunisation for Hepatitis B is expensive, and due to the cost requirement, most parents are not able to afford it. He again states in his response that Government intends to include it in the routine immunisation, and he goes further to give us timeline, which is the first quarter of next year.

Mr Speaker, I would want to find out from the Hon Minister what the analysis of the cost has been, and how its impact on the Health Insurance would be, considering the fact that we are still facing problems with the funding for the Health Insurance.

The Speaker: Hon Members, let me just read a Communication from His Excellency the President, dated 24th October 2025.

COMMUNICATION FROM THE PRESIDENT

Absence from Ghana



REPUBLIC OF GHANA

Jubilee House



The President

24th October, 2025

Rt. Hon. Speaker,

ABSENCE FROM GHANA

In accordance with Article 59 of the 1992 Constitution of the Republic of Ghana, I write to formally notify you of my intention to travel outside the country for an official working visit to Bamako, Mali on Monday, 27th October, 2025 and return same day.

Additionally, I will leave the country on Tuesday, 28th October 2025, for Paris, France, to participate in the 8th Edition of the Paris Peace Forum, scheduled to take place from 29th to 30th October 2025.

Following this engagement, I will depart Paris on 31st October, 2025 for Cairo, Egypt, to attend the opening ceremony of the Grand Egyptian Museum on 1st November, 2025.

I plan to return to Ghana on 1st November, 2025.

In my absence, the Vice President, Professor Naana Jane Opoku-Agyemang, shall perform my duties in accordance with Article 60 (8) of the Constitution.


JOHN DRAMANI MAHAMA

**THE RT. HON. SPEAKER
OFFICE OF PARLIAMENT
PARLIAMENT HOUSE
ACCRA**

cc: The Vice President
Jubilee House
Accra

The Chief of Staff
Jubilee House
Accr

The Speaker: Hon Minister, I am sure you heard the Hon Member loud and clear.

Mr Akandoh: Mr Speaker, since the Hon Member is veering into figures, I do not have the figures available at the moment, and he may file the appropriate Question, and I will come back with the figures. I do not have the figures available now.

The Speaker: Actually, he did not say the cost. He said what goes into the cost. So, he is looking for the ingredients; the various things that go into the cost. But if he asked how much it costs, that is specific, then you can give that response. But if he asked that what goes into it, it means you have to analyse and see the elements that come together to make the drug cost that much.

I hope I got you clear.

4:24 p.m.

Dr Mahama: Mr Speaker, my question was in two phases. The first part of the question was very clear as to the cost, and the second part was to the analysis of the cost in respect of the health insurance. So, if the Hon Minister is telling us that he does not have the figures and that he will provide this House with the figures, it is a very valid answer. But if he says that question will have to come by way of a substantive Question, I beg to disagree, because it is directly flowing from the answer he gave to Hon Mavis Nkansah-Boadu.

4:25 p.m. — [HON SECOND DEPUTY SPEAKER, MR ANDREW ASIAMAHO AMOAKO, IN THE CHAIR]

Mr Second Deputy Speaker: Yes, Hon Minister, are you saying you should be given time to come back?

Mr Akandoh: Yes, Mr Speaker, I will make the cost available to the House.

Mr Second Deputy Speaker: Very well. Let me go to Hon Nyindam.

Mr Matthew Nyindam: Mr Speaker, thank you. Looking at the birth rate and the cost involved in this country, which the Hon Minister admitted, what step is the Ministry taking from now to the first quarter of 2026, to assist pregnant women who are unfortunately carriers?

Mr Akandoh: Mr Speaker, immunisation is a structured programme under the Ministry, and so it was not part and as I speak now, it is even not part of our Immunisation Programme. But as part of the Free Primary Healthcare we intend to roll out, we are bringing that on board. So, for now, we do not have any ad hoc measure to deal with it. We are taking our time, and we want to put it as part of the traditional immunisation. So, for now, we do not have any programme for it. We are waiting for the substantive programme we have planned.

Mr Second Deputy Speaker: Very well.

Hon Isaac Boamah?

Mr Isaac Boamah-Nyarko: Mr Speaker, I think the Question the Hon Minister was asked is key because of the consequences of lack of immunisation in that regard. Given the response of the Minister to the effect that the consideration for national attention will be done by the end of the first quarter of 2026, I want to ask the Hon Minister if we have data now on where it is endemic. I think there should be a structured approach to see how best the Ministry would resolve or make special— He has come to this House with a special budget to deal with emergency situations. So, I want the Hon Minister to tell this House what special arrangements he has put in place from the Budget, if really he has received releases from the Ministry of Finance, and then make provision for the endemic places to be treated as a state of emergency. Mr Speaker, thank you very much.

Mr Akandoh: Thank you, Mr Speaker. I repeat, the Ministry has a programme for bringing on board the Hepatitis B birth dose and so I will indulge Hon Members to be patient and we will enrol it. We did not come to meet it at all, please, with the greatest of respect.

Mr Second Deputy Speaker: Thank you very much.

Hon Minority Leader?

Osahen Afenyo-Markin: Mr Speaker, according to the Hon Minister, Government will soon introduce immunisation to save lives. But what I find curious is the fact that, though Hepatitis B affects a chunk

of our population, in fact, records from Ghana Health Service gives data which is in excess of a million people being infected—I made a Statement on this on the floor of the House. They estimate almost 10 per cent of our population is infected by Hepatitis B. What is quite curious is the fact that we have free antiretroviral drugs for HIV/AIDS.

May I know from the Minister, in view of the effort being made for the immunisation to be free very soon, are there plans by the Ministry to also add Hepatitis B treatment to the category of HIV/AIDS? This is because they all depend on the antiretroviral for treatment. One effect of this Hepatitis B is that it leads to liver cancer and the deterioration is faster than the most feared HIV/AIDS. All over, people are suffering and it is not only through sex that people get infected. Even sweat as well as food and drinks. Body contact alone can cause the infection.

So, Mr Speaker, can the Minister make that disclosure if there is any such plan?

Mr Akandoh: Mr Speaker, my information is that it is very difficult to treat or to cure, if not impossible but then, when one gets the symptoms, the National Health Insurance, as we have it at the moment, covers that. And it could be managed—*[Interruption]* That is why I am saying that it is not curable. Its treatment is very difficult as it stands now.

Mr Speaker, having it as symptoms could be considered under the National

Health Insurance as we have it at the moment. Every disease has its own symptoms. For example, if one goes to the hospital and have headache, they try to manage the situation and those situations are under the National Health Insurance as we have it at the moment. But to say we are treating or curing Hepatitis B on the National Health Insurance, that, I will have to recheck.

Osahen Afenyo-Markin: Mr Speaker, with respect, I need to lay a good foundation for my respected Colleague, the Minister to understand the situation. When a person gets HIV and it is so tested and viral load is determined, the health authorities puts the person on antiretroviral drugs which is free. *[Interruption]*

Mr Second Deputy Speaker: Hon Leader, please address me.

Osahen Afenyo-Markin: It is free and I want Colleagues to pay attention to this matter because a lot of our constituents are being affected and we need to push for Government to take a position on this. The treatment and the management of Hepatitis B is also done through the same antiretroviral drugs but that one is not free.

4:34 p.m.

The same treatment, the same drugs that are used to manage HIV are the same drugs that are used to manage Hepatitis B. And I am saying that the data available at Ghana Health Service points to the fact that we have more Ghanaians being infected by Hepatitis B than HIV.

So, there is a public health situation here. Unfortunately, the treatment of Hepatitis B is not covered as well as the vaccination, but the Hon Minister has assured us of the immunisation bit. He has assured us that it will be done very soon.

What I want to know from my respected Colleague is that are there plans to put infected people also on the antiretroviral drugs for free? Because by the time the symptoms show, the person would have gotten to the advanced stage. Mr Speaker, I have done an exercise in Winneba because when the data came out, it was so alarming. And a lot of those who were infected were saved by Kwadwo Safo Kantanka's drugs. He had some liver boosters. Sometimes those who were on it went and took the test and the viral load would have come to the lowest level. Sometimes it could not be seen. It actually finds a way of getting out of your system according to the data and the report from the health professionals.

But Mr Speaker, because we do not have free treatment, people test positive and with liver infection, within six months to a year, they lose it and die. So, if the Hon Minister can assure us that Government will take steps to make the treatment of Hepatitis B free, the better it will be for our population, especially when the mode of transmission is quite high—the way people get infected and all. It is not just one way. So, it is a very important matter that our Colleague has brought to the Floor.

Mr Second Deputy Speaker: Thank you, Hon Minority Leader.

Mr Akandoh: Mr Speaker, the information I got from the experts indicate that the set of treatments for HIV is completely different from that which is used to handle Hepatitis B. Therefore, the two cannot be the same, with the greatest of respect. So, Mr Speaker, if it is the opinion of the Hon Minority Leader that we should really look at that which will handle Hepatitis B, then that is a different argument altogether, and we may decide to reflect on it.

Thank you.

Mr Second Deputy Speaker: Yes, Hon Minority Leader?

Osahen Afenyo-Markin: Mr Speaker, this period is not for debate, but once the Hon Minister put out some facts, it is important that a question is put for clarity. According to our respected Colleague, the Hon Minister, he says that according to some experts, the treatment or the management of Hepatitis B does not use —*[Interruption]*. I use the word antiretroviral. *[Uproar]*

Mr Second Deputy Speaker: Hon Members, we do not need to split our hairs over this. He is on the Floor. If you have a Question, you can file it. Do not worry. Let us allow him.

Yes, Hon Minority Leader, you may proceed.

Osahen Afenyo-Markin: Mr Speaker, is the Hon Minister in his Answer telling this House that the treatment of Hepatitis B—

Mr Second Deputy Speaker: Hon Minority Leader, please address the Chair.

Osahen Afenyo-Markin: Is the Hon Minister saying that the treatment for Hepatitis B does not rely on antiretroviral medication? Is that what he is saying? That is my question. *[Uproar]* Mr Speaker, with respect, I said this is a serious public health issue. That is some of the mistakes we make as political leaders. People in our constituencies are dying. The infection rate, according to Ghana Health Service, is about 10 per cent of the population. That is an alarming figure. So, if we are talking about 30 million, it means over 3 million of our populace are infected by Hepatitis B, far higher than HIV. So, let us not be partisan about this.

What I am saying is that for HIV, they rely on antiretroviral for treatment, and it is for free. So, unlike some 20 years ago where people get HIV and they die, today, because of the antiretroviral drug, people are able to take it religiously, manage their viral load, and they live a healthy lifestyle. I am saying that in the case of Hepatitis B, which kills faster, and leads to liver cancer, and by the time you would realise it, your liver would have been gone, vaccination is not for free. For adults, treatment is not for free. Meanwhile, it affects the population in larger numbers than the HIV that we fear. People are surviving more on HIV than Hepatitis B, and they are dying quietly.

All I want to know from the Hon Minister is that, would he take steps to put Hepatitis B treatment on the National

Health Insurance Programme and make it free as well?

Thank you, Mr Speaker.

Mr Second Deputy Speaker: Very well.

Mr Akandoh: Mr Speaker, that will be considered. Thank you.

Mr Second Deputy Speaker: Thank you. We turn to Question 7(b) and the Question stands in the name of Prof Titus Kofi Beyuo, Member for Lambussie. Hon Member, you have the floor now.

Steps to Ensure Decentralisation of Interventional Radiological Service

Prof Titus Kofi Beyuo (NDC—Lambussie): Mr Speaker, I beg to ask the Minister for Health the practical steps taken by the Ministry to ensure decentralisation of Interventional Radiological Service in the country to curb preventable deaths from stroke, heart attacks and other acute conditions.

Thank you.

Mr Second Deputy Speaker: Thank you very much.

Yes, Hon Minister?

Mr Akandoh: Thank you, Mr Speaker.

I thank the Hon Member for this important Question, which highlights a critical component of our national strategy to reduce preventable death

from cardiovascular and other acute medical conditions.

Mr Speaker, Interventional Radiology is a specialised branch of medicine that uses minimally invasive image-guided techniques to diagnose and treat various medical conditions, including blocked blood vessels, internal bleeding, and organ disorders. It plays a vital role in the rapid management of emergencies such as stroke, heart attack, and other life-threatening conditions.

Mr Speaker, the situation inherited by the Government was that only two public facilities were providing some level of interventional radiology and cardiac catheterisation services. In addition to few centres operating within the private sector, this limited distribution of services constrains equitable access and contributes to delays in the management of cardiovascular and emergency cases.

4:44 p.m.

To address this gap, the Ministry of Health, in collaboration with the Ghana Health Service and other implementing agencies, the National Ambulance Service and other development partners, has adopted a comprehensive plan to expand interventional and emergency services nationwide.

First, in the area of capacity building, the Ministry has partnered with the Ghana College of Physicians and Surgeons to establish a fellowship programme in Interventional Radiology (IR). Funding and scholarships have been secured to support training with

attention to equitable regional representation. Continuous professional development programmes have also been revised to include interventional techniques for radiologists, cardiologists and emergency physicians. Additionally, telemedicine platforms are being deployed to provide mentorship and technical support to regional hospitals.

Second, regarding infrastructure, the Ministry has prioritised the expansion of interventional and cardiac care facilities. Six regional hospitals are being upgraded over the next two years to include CT-guided intervention suites and portable ultrasound systems for emergency vascular procedures. Mr Speaker, most notably, we have set in motion processes for the establishment of three new catheterisation laboratories, known as Cath labs, in selected tertiary hospitals to support interventional radiology and cardiac procedures.

This represents a major advancement in the equitable distribution of advanced cardiovascular care. Our targets include increasing the number of certified interventional radiologists from four to 20, establishing functional IR services in at least six additional regions, and reducing the median door-to-procedure time for acute stroke and coronary syndromes to under 90 minutes.

Finally, Mr Speaker, sustainability is being ensured through the integration of IR-related maintenance and consumable costs into the National Health Insurance Scheme and by progressively expanding local training programmes to develop a fully Ghanaian faculty by 2028.

Mr Speaker, the Ministry of Health remains firmly committed to decentralising interventional radiology and cardiac intervention services as a key element of our national health strategy. Through targeted interventions in training infrastructure, including the establishment of three new Cath labs and strong partnerships, we are ensuring that life-saving, minimally invasive procedures become accessible to all Ghanaians, regardless of their region or residence.

Thank you.

Mr Second Deputy Speaker: Professor Beyuo, any supplementary question?

Prof Beyuo: Yes, thank you.

Mr Speaker, I ask this Question because not too long ago, we lost a colleague, a medical doctor, who needed catheterisation in the Ashanti Region and had to be flown to Accra. So my next question is—

Mr Second Deputy Speaker: Hon Member, when you say catheterisation, what do you mean?

Prof Beyuo: That is what the Minister talked about. It is an Interventional Radiological procedure that through a wire, guide into the vessel to remove a clot. So, the colleague had a clot in the blood vessel, and it was blocking the supply of blood to a critical part of his heart.

Mr Second Deputy Speaker: All right.

Prof Beyuo: And as we speak, this service is not available in any part of the country outside of Accra. So, even from Komfo Anokye Teaching Hospital, the person could not make it to Accra and passed away. There are people from my Constituency in Lambussie, people in the Upper West Region, and the Northern Region, who want to hear from the Minister on how soon this catheterisation lab can be made ready in other parts of the country.

But while at it, I commend the Ministry for the comprehensive package to decentralise these services so that Ghanaians everywhere stand a chance of surviving if they have an acute event. But I want to know, how soon can these labs be established? Thank you.

Mr Second Deputy Speaker: Yes, Hon Minister, how soon?

Mr Akandoh: Thank you, Mr Speaker.

Mr Speaker, as a matter of fact, the Government is at the concluding stage of the procurement processes. Therefore, I can assure you that very soon, three of the catheterisation or Cath labs will be established across various locations in the country.

The information I have is that, Mr Speaker, this equipment is not manufactured and put in the showroom for sale. It is based on order, which is the difficulty. But we are at the concluding stage of the procurement process, and I can assure you, very soon, it will be ready.

Mr Second Deputy Speaker: Thank you very much.

Yes, Hon Kabiru.

Dr Tiah Abdul Kabiru Mahama: Mr Speaker, I am not a medical doctor, and I provide this disclaimer because I am following from the response the Minister gave. Based on the background of this issue, the urgency and seriousness of that matter, I am sure the Minister will not be content with himself to say that very soon, without definite, certain or clear timelines. Even procurement has timelines.

Mr Speaker, I am sure the people of Lambussie will not be happy with the Minister's response that it would be procured very soon. The question was, how soon? And the response was that it would be very soon, Mr Speaker. I mean, that should not stand as an answer in this House. We want the Minister to provide the timeline, just like he did in the case of Hepatitis B, first quarter of next year. We want that certainty, so that people can plan their lives around that. And that is specifically the answer we are looking for. How soon? We want definite timelines, Hon Minister.

Mr Second Deputy Speaker: Yes, Hon Minister. Personally, until your explanation, I never knew that this was a very important Question. So, I share the sentiment of Dr Kabiru, if you can give us specific timelines.

Mr Akandoh: I entirely agree with you. It is an issue of emergency. His Excellency the President, has given clear

directives, and that is why I told the House that we are at the concluding stage of the procurement. But Mr Speaker, I have been following the development keenly, and just to share with you, I have already indicated that it is not manufactured and put at the showroom. It will take not less than two to three months to get a machine fully manufactured. And it is not being manufactured in Ghana. It will have to be on the sea to travel to the country. So, give or take, we are talking about not less than five months.

That is the explanation, Mr Speaker.

Mr Second Deputy Speaker: Yes. So, we will now turn to the Oral Questions. We will come to Dr Kingsley Agyemang, Hon Member for Abuakwa South, on the Question numbered 86.

Dr Agyemang, you have the floor now.

ORAL ANSWERS TO QUESTIONS

MINISTRY OF HEALTH

Coordination of Data for Evidence-Based Health Policies

Dr Kingsley Agyemang (NPP — Abuakwa South): Mr Speaker, I beg to ask the Minister for Health what approach the Ministry is taking to coordinate data collection, analysis and dissemination of research to generate evidence-based health policies.

Thank you.

Mr Second Deputy Speaker: Yes, Hon Minister.

Minister for Health (Mr Kwabena Mintah Akandoh): Mr Speaker, I thank the Hon Member for the Question, which highlights the importance of data governance and research in shaping evidence-based health policy.

The Ministry of Health recognises that healthcare is a data-intensive sector, and that timely, accurate, and secure data are essential for sound policy and effective service delivery. To this end, the Ministry is implementing a comprehensive strategy to strengthen coordination in data collection, analysis, and use across the health sector.

First, we are automating data capture at all levels of care to replace manual entry, reduce errors, and ensure timely reporting. This digital transformation enhances accuracy and efficiency while freeing health workers to focus on patient care.

4:54 p.m.

It also ensures that our systems are robust, interoperable, and locally managed, so the Ministry maintains full control of medical data without dependence on external vendors.

Second, we are establishing a National Health Database (NHD) under the Health Information System Strategic Plan to integrate data from the Ghana Health Service, the National Health Insurance Authority, and other agencies

into one reliable repository for policy planning, and research.

Third, we are harmonising health data and metadata across all 29 agencies to ensure consistency and interoperability, and are collaborating with the Ghana Statistical Service to align health data with national demographic information for more accurate analysis. The Ministry is also partnering with universities and research institutions to apply advanced analytics and big data tools, enabling real-time insights for planning, resource allocation, and performance monitoring. Complementing this is the Knowledge Translation Platform, which ensures research findings are systematically synthesised into actionable policy recommendations.

Mr Speaker, through these efforts, the Ministry is building a modern, secure, and integrated health information ecosystem that supports real-time decision-making, safeguards data sovereignty, and reinforces our commitment to evidence-based health policy.

Mr Second Deputy Speaker: Thank you very much. Yes, Dr Agyemang, any further question?

Dr Agyemang: Thank you very much, Hon Minister. I might have missed it. Forgive me if I did. My question was in three folds; collection, analysis, and dissemination. Dissemination is very key, but in your response, I did not hear a response on dissemination of this data. Could you please point to the

various paragraphs that answer the question on dissemination? Thank you.

Mr Second Deputy Speaker: Yes, Hon Minister?

Mr Akandoh: Mr Speaker, if the Hon Member will come again. Dissemination, as in the analysis we get from the data gathering, what we learn from it and disseminating that information; what does the Hon Member exactly mean by “dissemination”?

Dr Agyeman: Thank you very much. You have collected data and analysed the data. How do you disseminate that data to transform into policy evidence or evidence policies that we would require in the health sector? Thank you.

Mr Second Deputy Speaker: Yes, Minister?

Mr Akandoh: Mr Speaker, what data does to policy is — Mr Speaker, I will give you a practical example. It is out of data that we realised that about 43 per cent to 45 per cent of our mortalities in this country are attributable to Non-Communicable Diseases (NCDs); It is data. So once you have this data as a government or as a leader, it informs your policy, hence the establishment of the Ghana Medical Trust Fund to deal with the issues. So, when you know that there is a problem out of the data, maybe in some cases, you have to embark on sensitisation drive. For example, if you have a data on outbreak, then you have to roll out your public health strategies to sensitise the people on how they can prevent themselves from that particular

outbreak. That is how the analysis or the data collection informs decision makers to be able to govern well.

I thank you for the opportunity.

Mr Second Deputy Speaker: Yes, Dr Agyemang?

Dr Agyemang: Thank you very much, Mr Speaker. I think I will engage further on the response given on the dissemination. However, a quick follow-up. Again, in consonance with best global practices, you would realise that research calls are usually announced so that we reduce bias and other issues that come with it. The response, however, was lagging on strategies to reduce biases to research. Can the Hon Minister consider such issues that are usually in line with best practices when it comes to research calls? Thank you.

Mr Second Deputy Speaker: Yes, is it a question or you—

Mr Akandoh: It is an admonishment.

Dr Agyemang: It is not an admonishment. The question is, what strategies have they put in place to mitigate research bias with regard to best practices on research calls?

Mr Second Deputy Speaker: Yes, Hon Minister?

Mr Akandoh: Mr Speaker, there are various platforms for advertising or disseminating the outcomes of research. For me, every research has a reason. One has a target. If the target at the end of the

day is to create some kind of awareness, at the end of the research, one does so given the aims for the research. But if the research is also to guide one to take certain decisions, one also does accordingly. But that notwithstanding, the issue of best practice is also taken on board. Usually, we use platforms like the Health Summit, the World Health Assembly, and other avenues or other fora to do the dissemination. Thank you for the opportunity.

Mr Second Deputy Speaker: Hon Members, let us turn to Question 95, and the Question stands in the name of the Hon Member for Old Tafo.

You wanted further questions? All right, then let me give it to the Hon Member for Oforikrom. Dr Kabiru Mahama, you have spoken a lot. Let me give it to the Hon Member for Oforikrom.

Mr Michael Kwasi Aidoo: Thank you very much, Mr Speaker, for this opportunity. I want to ask a question as a follow-up to the answer given by the Hon Minister. The Minister stated that the data that is captured is very relevant to the health sector. But currently, I am privy to information that the National Electronic Medical Record System is down. So, if the National Electronic Medical System that captures all records of medical issues is down, how is the Hon Minister capturing the data? How is he able to store the data and even access and use that data for the benefit of Ghanaians?

Second Deputy Speaker: Very well. Yes, Hon Minister?

Mr Akandoh: Mr Speaker, it is true that we have some challenges with regard to our patient electronic records. I can assure the Hon Member that tomorrow, I am to tell the whole country what we have inherited, what we are going through, and most importantly, the solution. I can assure the Hon Member that it is not going to block or hinder the collection of data at all in any case. It will not.

Thank you very much.

Mr Second Deputy Speaker: Very well. So we will turn to — Dr Kabiru Mahama. Look, he is your friend, though; I am saying Mr Akandoh is your friend.

All right, let us hear you.

Dr Mahama: Mr Speaker, I am rather difficult because yesterday at a TV programme, it was a serious issue on the matter of the contract between the Minister for Health and the Lightwave Health Information Management System.

5:04 p.m.

The fact that we are now going back to manual system of collecting data, that after an initiative by the previous government to digitalise our data and to contract to ensuring that we have electronic collection of patient data for the purposes of coordinating and informing evidence-based health policies, we are now back to that era. That is the reason I want the Minister to rather take this issue now more importantly because the MPs, the representatives of the

people, are here. If he is going to organise a press conference somewhere to address them, I am not sure I will be at the press conference and I am not sure how I am going to tell the people of Walewale how the Minister is addressing this issue.

Mr Speaker, the fact is that these people have not been paid their money. What measures is the Minister putting in place to pay them their money so that they can bring back their services? Mr Speaker, this is my question to him.

Mr Second Deputy Speaker: Hon Minister, he is saying this is the biggest platform you can have.

Mr Akandoh: Mr Speaker, I agree. I will take up the challenge, so he should hear me. In 2019, the Government of Ghana signed an agreement with a company called Lightwave eHealthcare Solutions. The contract sum, which was US\$100 million was to connect 950 health facilities in the whole country. The contract was supposed to expire in 2022. In 2022, the contract was extended twice to expire on 31st December, 2024. At the end of the 2024, out of the 950 facilities, only 450 had been connected. Out of the US\$100 million, US\$77 million had been paid; that is more than 77 per cent, meanwhile, less than 50 per cent of the job had been done.

Mr Speaker, as part of the contract, it was supposed to be divided into what we call milestones. So, at the end of each milestone, the infrastructure and everything must be handed over to the state. It is unquestionable for any

government appointee to sign an electronic agreement where an individual will be hosting our data but the state will not have access to our health data and servers. That is what we inherited. When we took office in 2025, the contract had expired. So, the issue of the Government terminating any contract does not arise, because the contract actually expired on 31st December, 2024. So, there was no issue of termination.

Mr Speaker, when we took office, as part of the contract, we said they should hand over whatever they had done to the state. Before we sat to take delivery of whatever had been done, we needed, as a Ministry and Government, to know whatever we were inheriting, so a forensic audit was conducted. As part of the forensic audit, what we realised was that there were gaps in the hardware and infrastructure alone. For example, it was stated in the agreement to supply Hewlett-Packard (HP) laptops, but we went to the facilities and a different brand had been supplied. They were asked to supply 20, but we went to the grounds and less than 20 had been supplied. So, there were issues of quality and quantity. When we analysed the gaps for the hardware alone, they were over US\$18 million.

Mr Speaker, it will interest one to know that in the next milestone, Ghana's Government paid more than US\$10 million to the vendor, and he was supposed to connect about 157 facilities. Guess what? Only seven were verified. After all this, we said that, because we do not need any disruptions, we were ready to sign another agreement because there

was no agreement or contract. It does not also make sense for the software of the country's data to be owned by an individual, so we were ready to sign a licence renewal agreement for two years. The vendor indicated that in the agreement, he does not want to hear anything about data. Yes. The vendor again indicated that we should write, as part of the contract, that we, the Ministry, has verified 461 instead of 150. There and then, we put our heads together as a Ministry to find a solution to the matter than lament.

So, the good news is that, I have shared with Hon Members, the challenges we inherited, where the electronic medical records were hosted by an individual and the country did not have access to our servers. But at the moment, we have the solution and we are rolling out in the whole country. We have a four-week plan. In the first week, which is next week, we are rolling out more than 100 facilities. In the third week, we will roll out more than 100 facilities and I can assure Hon Members that within the shortest possible time, we will get all these 450 facilities covered.

Mr Speaker, what this vendor was doing to the state was that, as and when he demanded, he would switch the server off. Mr Speaker, for your information, the server does not even sit in Ghana. It is far away in India. We are religiously guided by this contract and we will put in place the necessary solutions so that we will prevent the reoccurrence of the mess we have inherited. As I have already indicated, tomorrow we will give a full presentation on these issues, pictorial

evidence, and also tell the people of this country what we are going to roll out. I thank you for the opportunity.

Mr Second Deputy Speaker: Thank you very much. We turn to Question 95.

Dr Mahama — *rose* —

Mr Second Deputy Speaker: Dr Mahama, you do not own the Question. I gave you the opportunity to ask a supplementary question. The question has even revealed some of the information, because you insisted that this is the biggest platform, and he alluded to that. So, let us move on. Question 95 and it stands in the name of Hon Vincent Ekow Assafuah, Old Tafo Constituency.

Timelines for Completion of Tafo Government Hospital Maternity Block

Ms Gloria Owusu on behalf of Mr Vincent Ekow Assafuah (NPP — Old Tafo): Thank you, Mr Speaker. In accordance with Order 88 (3) of our Standing Orders, I rise with authority from the Hon Vincent Ekow Assafuah, Member of Parliament for Old Tafo, to ask the Minister for Health the timelines for the completion of the Maternity Block in the Tafo Government Hospital.

Mr Akandoh: Mr Speaker, I wish to inform this august House that both Executive and Parliamentary approvals were duly secured for the financing and supply contracts related to the Tafo Maternity Hospital project. Additionally,

value for money (VfM) negotiations were successfully concluded.

However, Mr Speaker, as we may recall, in January 2023, the Government, through a circular issued by the Ministry of Finance, suspended payments on most externally funded projects. As a result, Deutsche Bank AG, the project's financier, was compelled to halt the necessary processes required for project effectiveness. Consequently, the project has been temporarily put on hold due to the debt restructuring.

Mr Second Deputy Speaker: Yes, Hon Gloria, any supplementary questions?

Ms Owusu: Yes. Mr Speaker, has the Ministry of Health engaged the Ministry of Finance to find a new funding or support to complete the Tafo Maternity Block and other health projects?

Mr Akandoh: Mr Speaker, we are still pursuing the same source of funding, but it has to do with the debt restructuring. So, till we sign the bilateral agreements with them or conclude on the restructuring, we think that we have not hit the rock yet to be able to change course. It is due to the debt restructuring, so we are still pursuing the source of funding.

5:14 p.m.

Ms Owusu: Mr Speaker, what temporary steps has the Ministry taken to reduce overcrowding and lack of staff storage [sic] at the Tafo Government

Hospital, where the maternity block remains suspended.

Mr Second Deputy Speaker: Yes, Hon Minister?

Mr Akandoh: Mr Speaker, I thought we were talking about construction of maternity block. We are now veering into the issues of recruitment of workers.

Mr Second Deputy Speaker: But she is talking about overcrowding.

Mr Akandoh: Mr Speaker, what I can assure the Hon Member is that we have a programme for that facility and we will go according to the programme.

Mr Second Deputy Speaker: Hon Gloria?

Ms Owusu: Mr Speaker, please, I want to find out from the Minister how soon?

Mr Second Deputy Speaker: Yes, Hon Minister?

Mr Akandoh: Very soon.

Mr Second Deputy Speaker: First Deputy Minority Whip?

Alhaji Habib Iddrisu: Mr Speaker, with all due respect to the Minister, I think the answer he has provided is completely inadequate. Minister, you can do better. Kindly respond to the question. Your response is inadequate.

Mr Second Deputy Speaker: Minister, the Leader is your Friend—

Alhaji Habib Iddrisu: Mr Speaker, he is my former Friend, so he should be able to —

Mr Akandoh: Mr Speaker, this House is a House of records and I do not expect that my Colleagues would want me to lie to the House. This project is dependent on the conclusion of the debt restructuring. I must know when the debt restructuring will end before I can tell you when we can even have access to the sources of funding. So, Mr Speaker, we are pursuing it, but with the greatest of respect, I do not want to be called to appear before the Committee on Assurances. Therefore, I am being very careful here.

Mr Second Deputy Speaker: Thank you very much. Hon Members, this is a constituency-specific Question, so we will not roll-out any supplementary question to any Member.

We will turn to Question 262. Hon Member for Suhum, Mr Frank Asiedu Bekoe.

Expansion and Refurbishment of Emergency Ward at Suhum Government Hospital

Mr Frank Asiedu Bekoe (NPP—Suhum): Mr Speaker, I beg to ask the Minister for Health the plans of the Ministry for the expansion and refurbishment of the emergency ward of the Suhum Government Hospital.

Mr Second Deputy Speaker: Hon Minister?

Mr Akandoh: Mr Speaker, thank you for the question. The Ministry fully acknowledges the essential need for a well-functioning and adequately equipped emergency ward at the Suhum Government Hospital. At present, the expansion and refurbishment of the emergency ward have not been captured under the 2025 fiscal year budget. However, the Ministry intends to undertake a detailed assessment of the facility to determine the scope of work required. Based on the outcome, the project will be considered for inclusion in the Capital Expenditure (CAPEX) programme in the near future.

Mr Speaker, I wish to assure this august House that the Ministry will prioritise this intervention to strengthen emergency healthcare services for the people of Suhum and its surrounding communities.

Thank you.

Mr Second Deputy Speaker: Your choice of words; ‘in the near future’, meanwhile, the 2026 Budget is coming. Why do you not say 2026 Budget? Anyway, Hon Member for Suhum?

Mr Bekoe: Mr Speaker, nobody should think that that facility belongs to only Suhum people. Because when you travel from Accra to Kumasi, you will pass through Suhum. Those from Akosombo, Koforidua, going to Winneba-Cape Coast, they pass through Suhum.

Between January and September 2025, we have had 263 reported accident

cases in Suhum. So, I want the Minister to assure us, if he is going to fast-track his assessment of the situation in Suhum. Because the Suhum Government Hospital serves a lot of people, including people who live outside Suhum. Can the Minister assure us of some assistance?

Mr Second Deputy Speaker: Yes, Hon Minister?

He has given the importance of Suhum Government Hospital.

Mr Akandoh: Mr Speaker, thank you. I wish to reiterate some aspects of my response, and I quote,

“... the Ministry intends to undertake a detailed assessment of the facility to determine the scope of work required. Based on the outcome, the project will be considered for inclusion...”

Mr Speaker, I think what the Hon Member needs to do is to work with the Ministry so that we can assess the situation and see what we can do.

Thank you.

Mr Second Deputy Speaker: Very well.

Hon Asiedu Bekoe, at times it is better to deal with the Minister one-on-one. So, work with him. Let me take your last question.

Mr Bekoe: I wish to state that in trying to collaborate with you, I should meet you in the office.

Currently, if there are accident cases, one will go and find patients lying on the floor. Can the Minister come in with some temporary measures to ameliorate the situation in Suhum?

Mr Second Deputy Speaker: Yes, Hon Minister?

Mr Akandoh: Mr Speaker, I fully agree with the Hon Member that the Suhum Government Hospital is at a very strategic position that serves anybody who plies the road. And so, as I have already indicated, he should work closely with my office, and I can assure him that we are in to serve the country. We will do our best. Thank you.

Mr Second Deputy Speaker: Thank you very much. Hon Members, we will turn to Question 263.

Hon Member for New Juaben North,
Mr Nana Osei Adjei.

Status of the Eastern Regional Hospital

Mr Michael K. Aidoo *on behalf of Mr Nana Osei-Adjei (NPP — New Juaben North):* Mr Speaker, I beg to ask the Minister for Health the status of the Eastern Regional Hospital which has been abandoned after 70 per cent completion.

Mr Second Deputy Speaker: Yes, Hon Akandoh?

The way you are smiling, it means you are going to give a straight —

Mr Akandoh: Mr Speaker, the Design, Construction and Equipping of the Eastern Regional Hospital was awarded in May 2020. The project made progress to attain a completion of about 57 per cent. However, the Project stalled due to the debt restructuring.

Mr Speaker, as indicated earlier, the Ministry of Health, in collaboration with the Ministry of Finance (MoF), will consider resuming once the process for the debt restructuring is concluded. So, Mr Speaker, if the project was abandoned, it was abandoned long ago, but we will go back to site and do the needful.

Thank you.

Mr Second Deputy Speaker: The way you were smiling, I thought you were coming to give a different response.

Yes, Hon Member for Oforikrom?

Mr Aidoo: Mr Speaker, no further questions.

Mr Second Deputy Speaker: Very well, thank you.

5:24 p.m.

Hon Members, we turn to the Question numbered 326. Dr Thomas Winsum Anabah, Hon Member of Parliament for Garu, you have the floor now.

Measures Taken to Complete the Garu District Hospital

Dr Thomas Winsum Anabah (NDC — Garu): Thank you, Mr Speaker. I rise to ask the Minister for

Health the measures the Ministry is taking to complete the Garu District Hospital.

Mr Akandoh: Mr Speaker, I wish to inform this honourable House that the Garu District Hospital Project in the Upper East Region forms part of the Government's flagship Agenda 111 initiative. The project had achieved an overall physical progress of 63 per cent as of December 2024.

Following Government's decision to rationalise the implementation framework of the Agenda 111 programme, the Ministry of Finance, by a letter dated 13th February, 2025, formally advised the Ministry of Health to incorporate all Agenda 111 projects into its annual budget proposals from 2025 onwards.

Subsequently, His Excellency the President, through a directive dated 20th February, 2025 (Ref. No. OPS 152/25/763), instructed that the oversight and implementation of the Agenda 111 projects be formally transferred to the Ministry of Health.

In line with this directive, Government commissioned an audit of all ongoing Agenda 111 sites, which is currently being undertaken by Messrs Architectural and Engineering Services Limited (AESL). The Ministry is presently awaiting the submission of AESL's final report and accompanying recommendations to inform the next course of action towards the continuation and completion of the Garu District Hospital Project. Thank you.

Mr Second Deputy Speaker: Dr Anabah, any further questions?

Dr Anabah: Mr Speaker, thank you very much. Let me thank the Hon Minister for his Answer. But I would like to explain that the Garu District Hospital is very necessary to be completed, simply because Garu has never had a district hospital. We used to patronise the Bawku Hospital, but due to the conflict and crisis in Bawku, we no longer have a hospital where we go to seek health. That is why the Government decided to put up the Garu District Hospital and as it has stalled. Most of the staff who were in the Bawku Hospital have been moved to either Zebilla, Binduri, Garu or Tempene. But if we check around the Kusaug area, out of the six districts, none of them has a district hospital. So, we think that completing the Garu Hospital is very imperative to ensure that at least it caters for the other districts or even helps reduce the burden and pressure on the Bawku Hospital if it resumes work. So, I would like to plead with the Minister to ensure that when they are considering hospitals to be completed, at least the Garu District Hospital should be part of those few hospitals. Thank you.

Mr Second Deputy Speaker: Thank you very much, but this is not a question.

Dr Mahama, this is not your constituency.

Dr Mahama: Mr Speaker, the response from the Minister goes beyond the specific constituency. And if the response of the Minister is far and wide, it is only well and good that we are

allowed to ask questions within the response of the Minister.

Mr Speaker, I would think that Garu was one of the many districts without a hospital and H.E. Nana Addo Dankwa Akufo-Addo's vision to make sure that every district— He mentioned that none of the districts in that particular enclave has a district hospital but through the Agenda 111, all of them would have had district hospitals.

Mr Speaker, the problem with Garu—I know Garu personally and I know the specific hospital. In fact, of all the hospitals in the Upper East Region, Garu District Hospital is one of the hospitals that have gotten at least a considerable percentage of work done. The work done on that particular hospital means that—

Mr Second Deputy Speaker: Hon Member, just ask the question.

Dr Mahama: Mr Speaker, I would like to find out from the Minister, the goal of this so-called audit that is to be done on the Agenda 111 and how soon are we going to get the audit done? What is the goal because work is being done?

Mr Second Deputy Speaker: Hon Mahama, respectfully, you are changing the question entirely. I hate to do that, but if you are asking questions directly to Garu, that would be alright. But now you are shifting to auditing among others.

Dr Mahama: Mr Speaker, the Hon Minister mentioned in his response that they have engaged an auditing firm to do

an audit of the Agenda 111 projects. I did not introduce that, Mr Speaker.

Mr Second Deputy Speaker: Hon Minister, could you tell us something about the intent of the audit?

Mr Akandoh: Mr Speaker, as one takes office and there are ongoing projects, one needs to understand where the contractors have got to and how much has been paid especially when there is not a dedicated source of funding for the projects. And also, Government strategises to get money to continue all these projects. So, Mr Speaker, one must understand what one has inherited and that will inform the strategy for securing funding for the project.

Mr Speaker, let me assure this House, that His Excellency the President has indicated time and again, that he is not going to abandon any ongoing projects that he has inherited. Therefore, I can assure the House that we will tackle any project that we have inherited in due course.

Mr Second Deputy Speaker: Thank you very much. We turn to the Question numbered 492.

Dr Anabah?

Dr Anabah: Mr Speaker, thank you very much. Once this is the house of records, I want to correct misinformation. Garu had a project from the National Democratic Congress (NDC) Government in 2012 by NMS Infrastructure Limited to build a hospital, so to say that Garu never had a hospital

— there was a project ongoing. The New Patriotic Party (NPP) Government came and stalled it and put it in the infrastructure tracker that they were building a hospital in Garu, which was not true. Thank you.

Mr Second Deputy Speaker: Hon Dr Anabah, respectfully.

Yes, Leader?

Mr Ayariga: Actually, I was outside when I heard the follow-up question about Garu, so I rushed in because I thought that there was some misrepresentation that needed to be corrected and I think that is what the Hon Member for Garu is trying to do. The questioner actually mentioned Garu as a beneficiary of Agenda 111, but that is not the case. The Garu Hospital was started by the NDC about nine years ago or even more. Then when the NPP took over, they stopped the project. So, for eight years, nothing happened there. Materials that were mobilised at site were all vandalised and stolen until NDC has come back. So, that is why he is asking the Minister what is going to be done to complete that hospital and whether it will be prioritised. So, when the Hon Member does a follow-up question and then presents it as if Garu was a beneficiary of Agenda 111, I think that the record needs to be corrected that Garu was not a beneficiary of Agenda 111. Bawku was not a beneficiary of Agenda 111.

5:34 p.m.

I think that the record needs to be corrected. Garu was not a beneficiary of

Agenda 111. Bawku was not a beneficiary of Agenda 111. I have not seen Agenda 111 in Pusiga. I have not seen one. I only know that because the NPP won in the Binduri Constituency, that is the only place that was assigned an Agenda 111 hospital; the rest did not get any Agenda 111 hospital.

Mr Second Deputy Speaker: Hon Majority Leader.

Alhaji Habib Iddrisu: Mr Speaker, I think the Hon Majority Leader did not get the right briefing. Because if you look at page 7 of the *Order Paper*, from the Answer to Question 326, the Hon Minister admitted that it is an Agenda 111 project. The Hon Minister said:

“Mr Speaker, I wish to inform this honourable House that the Garu District Hospital Project in the Upper East Region forms part of the Government’s flagship Agenda 111 initiative.”

Mr Second Deputy Speaker: Hon Members, Order!

Whose microphone is on?

Mr Ayariga: All right.

Mr Speaker, if the NPP Government decided to adopt that project—*[Interruption]*—Please, let us listen to each other. The Hon Minister is seated here. I know the hospital. The Hon Member of Parliament for Garu is seated here; he knows the hospital. And the Hon Minister’s records will show that this is a hospital project that was started under

the previous NDC Government. It was stopped by the NPP Government. Then after COVID-19, when NPP Government decided to embark on the Agenda 111 projects, they may have added Garu, but nothing was done there—[Uproar]

When the former President announced Agenda 111 hospitals, they were supposed to be new hospitals to be constructed. We all listened to him during the COVID-19 era when he announced that the Government would construct new hospitals from scratch. When they decided to adopt the hospital at Garu, then they had moved from your original objective and the Agenda 111. I am saying that I would have been happy if after adopting it, they had actually completed the hospital. But even after adopting it, they failed to complete the adopted hospital.

Mr Second Deputy Speaker: Hon Majority Leader, thank you.

But I think if there is any confusion, probably it might have emanated from the Hon Minister's response that it was part of the Agenda 111 hospitals.

Mr Matthew Nyindam: Mr Speaker, we should not be too diplomatic with the Hon Majority Leader. You see, the Hon Minister—

Mr Second Deputy Speaker: Am I being diplomatic to you?

Mr Nyindam: You are being too diplomatic with the Hon Majority Leader.

The Hon Minister is seated there. Mr Speaker, even before the Hon Majority Leader walked in, the questioner had admitted that apart from Garu, all the others had district hospitals. By then, the Hon Majority Leader was not here. And if you look at the Answer, the Hon Minister admitted clearly that it is an Agenda 111 hospital. But unfortunately, the Hon Minister, for some reasons, decided to allow the Hon Majority Leader to consume him. You do not do that, Mr Minister, you are in charge. The Ministry is in your hands. It is just the Chamber that is in the hands of the Hon Majority Leader. Do not let him bully you. This is an Agenda 111 hospital.

Mr Speaker, 63 per cent of the project is done which the Hon Minister had admitted. I am not debating. He is my Friend and I do not want the Hon Majority Leader to consume him. Do not allow yourself. Let the records reflect that it is an Agenda 111 hospital.

Thank you, Mr Speaker.

Mr Second Deputy Speaker: Thank you very much.

We will now turn to Question 492 in the name of the Hon Member for Ejisu, Mr Kwabena Boateng. Hon Member, take the Floor now.

Establishment of a Nursing Training College in Ejisu

Mr Kwabena Boateng (NPP — Ejisu): I beg to ask Minister for Health whether there is any plan on paper by

Government to establish a nursing training college in the Ejisu Municipality.

Thank you, Mr Speaker.

Mr Second Deputy Speaker: Yes, Hon Minister?

Mr Akandoh: Mr Speaker, I thank the Hon Member for the Question.

The Ministry of Health recognises the importance of expanding training opportunities to meet the increasing demand for skilled nurses and midwives. However, at present, there is no official plan or approved documentation for the establishment of a Nursing and Midwifery Training College in the Ejisu Municipality.

Mr Speaker, the Ministry currently oversees 94 Nursing and Midwifery Training Colleges across the country, 14 of which are located in the Ashanti Region. Before any new institution is established, the Ministry, through the Human Resource for Health Development Directorate, conducts a comprehensive needs assessment and feasibility study. These assessments take into account regional equity, availability of clinical training sites, financial and staffing considerations, and the accreditation requirements of the Nursing and Midwifery Council and the Ghana Tertiary Education Commission.

Mr Speaker, while the Ministry has received expressions of interest from stakeholders within the Ejisu Municipality, no formal proposal has yet been submitted or approved through the Ministry's internal processes.

That said, the Ministry remains open to collaboration with Metropolitan, Municipal and District Assemblies, as well as development partners, to explore the establishment of additional training institutions in underserved areas as the need arises, particularly for post-basic and specialist nursing programmes.

Mr Second Deputy Speaker: Yes, Mr Kwabena Boateng, any supplementary question?

Mr Boateng: Mr Speaker, yes.

Mr Speaker, by way of laying the basis for my next question, I would like to say that the Ejisu Municipality is a youthful population. The size is about 74,000 people. Mr Speaker, we have about eight senior high schools churning out graduates, and some of them may want to pursue a career in nursing. The Municipality is also an urban one, hosting about 46 major towns. The 2024 population projection was about 196,000.

Mr Speaker, I would want to find out from the Hon Minister if these are not imperatives which may warrant an establishment of a nursing training college in the Municipality, looking at the indices that I have enumerated.

Thank you.

Mr Second Deputy Speaker: Hon Minister, have you heard the imperatives? They are very necessary.

Mr Akandoh: Mr Speaker, all the factors he has enumerated are all very necessary requirements.

Thank you.

Mr Second Deputy Speaker: Hon Minister, I did not get your answer well. He is your Friend.

Mr Akandoh: Mr Speaker, the questioner only asked whether the factors he has enumerated are necessary for the establishment of a nursing training college. Is that not the question? I said yes, they are.

Mr Second Deputy Speaker: So, you are going to establish one?

5:44 p.m.

Mr Akandoh: Mr Speaker, are you asking the question for him? That was a very simple question I was supposed to respond to.

Mr Speaker, I will read some portions of my response, which states that the Ministry remains open to collaborating with Metropolitan, Municipal and National Assemblies as well as development partners to explore the establishment of additional training institutions, especially in underserved communities.

So, Mr Speaker, we will link up and then we will take the conversation from there.

Mr Second Deputy Speaker: Hon Member, do you still wish to ask another question?

Mr Boateng: Yes, Mr Speaker.

My last question may be—Mr Speaker, permit me to lay the basis. I am rightly informed that papers have been sent to the Ministry, as the Minister also rightly indicated that there have been some proposals. I want to know how long it will take for steps to be taken to consider setting up a nursing training in the Municipality.

Mr Second Deputy Speaker: Hon Member, in this House, at times it is better to see the Minister one-on-one. You have laid the basis. I will ask you to deal with the Minister personally. That may yield the needed results. So, after laying the basis, I will urge you to see the Minister and discuss the matter.

Mr Boateng: Very well, Mr Speaker. I take a cue.

Mr Second Deputy Speaker: Very well.

Hon Members, let us take the last Question from Hon Member for Hemang Lower Denkyira, Hon Lawrence Agyinsam.

Plans to Furnish the Twifo Hemang Lower Denkyira Health Directorate

Mr Lawrence Agyinsam (NPP—Hemang Lower Denkyira): Mr Speaker, I beg to ask the Minister for Health the plans of the Ministry to furnish the Twifo Hemang Lower Denkyira District Health Directorate with an administration block and staff bungalows to operate effectively and efficiently.

Mr Akandoh: Mr Speaker, the Ministry acknowledges the importance of providing a conducive working environment and adequate residential accommodation to enhance the effective and efficient operations of the Twifo Hemang Lower Denkyira District Health Directorate.

As part of the ongoing health sector reforms, the construction of office and ancillary facilities at the district level has been decentralised and now falls under the purview of the respective District Assemblies.

The Ministry will, however, collaborate closely with the Ministry of Local Government, Chieftaincy and Religious Affairs, as well as the District Assembly, to ensure that appropriate office and residential accommodation are provided to facilitate the effective performance of health staff within the District.

Mr Second Deputy Speaker: Yes, Hon Agyinsam, any further questions?

Mr Agyinsam: Mr Speaker, in the Minister's Answer, he stated that the construction of office and auxiliary facilities have been decentralised. When was it decentralised and what are the steps that I need to take to be able to engage the District Assembly so that Twifo Hemang Lower Denkyira would have the needed office?

Mr Second Deputy Speaker: Yes, Hon Minister.

Mr Akandoh: Mr Speaker, as far as I can recall, for the past eight to nine years, the Ministry of Health has not constructed district health directorate offices in the country. It has always been part of the projects by the District Assemblies. So, when they do the constructions, then we will fill in the staff.

Mr Agyinsam: Mr Speaker, my last question. If indeed the Minister says that it has to be done by the District Assembly, my follow-up question is, what about logistics, computers, and other things that need to be provided to the health directorate? Is it also to be provided by the assemblies or by the Ministries?

Mr Akandoh: Mr Speaker, as the Directorate falls under the Ministry, as and when they need assistance, they have the right to write to the Ministry or the Director General of Ghana Health Service, and we will look at it and consider it accordingly.

Mr Second Deputy Speaker: Hon Members, let me thank the Minister for attending upon the House to respond to Members' Questions on issues relating to health in their constituencies.

Hon Minister, we thank you, and you are hereby discharged.

Leadership, we are on Statement.

Ms Comfort Doyoe Cudjoe: Mr Speaker, I think we have two Statements.

Mr Second Deputy Speaker: I only have one, and for that Statement, I decided on Friday to take it today. The Statement is in respect of the Ghana Cedi at 60. I think that today, the Bank of Ghana celebrated and launched it.

So, it will be appropriate to ask the Chairman of the Committee on Economy and Development to read this Statement. Hon Afful, you may take the Floor.

STATEMENT

Ghana Cedi at 60

Dr Eric Afful (NDC — Amenfi East): Mr Speaker, let me thank you so much for the opportunity to read this Statement to commemorate the 60 years of our Ghana cedi since its introduction in Ghana.

Mr Speaker, the cedi continues to be durable, portable, divisible, uniform, limited in supply and generally accepted in Ghana. The cedi continues to function as a medium of exchange of goods and services, and also serves as a store of value, allowing people to save wealth over time. It also functions as a unit of accounts and values. This enables us to compare the worth of different items.

Mr Speaker, let me use this opportunity to commend the Government and the Governor of the Bank of Ghana and his team for their efforts to end *dollarisation*, that is, the practice of pricing goods and services in U.S. dollars instead of Ghana cedis in Ghana, which has a tendency to depreciate the Ghana cedi due to the increase in demand for the US dollar.

To be specific, the Government has issued a directive requiring all Government contracts to be denominated in Ghana cedis rather than in hard currencies such as the U.S. dollar.

5:54 p.m.

Moreover, the Governor of the Bank of Ghana (BoG), Dr Johnson Pandit Asiamah, strengthened these measures by mandating that all goods and services be priced solely in Ghana cedis. All these measures have contributed to making the cedi one of the strongest-performing currencies in the world, evidenced by its continuous appreciation from about GH¢17.00 to the US dollar in December 2024, to a current range of GH¢10.30 to GH¢12.00 to a dollar.

Mr Speaker, the Ghana cedi has undergone a remarkable transformation since the country gained independence, reflecting the nation's political, economic, and social changes over time. Its evolution tells the story of Ghana's journey towards economic sovereignty and stability in the face of both internal and global financial challenges. Before independence in 1957, the Gold Coast, now Ghana, used the British West African Pound, shilling, and penny, which were issued by the West African Currency Board (WACB). These colonial-era currencies served several West African territories under British rule. However, after achieving independence, Ghana sought to establish its own monetary identity as part of its broader goal of national and economic independence.

In 1958, Ghana introduced its first national currency, the Ghana Pound (£G), which was equivalent in value to the British pound sterling. The establishment of the Bank of Ghana (BoG) in 1957 played a vital role in managing this new currency. The introduction of the Ghana Pound symbolised the country's first step toward economic self-determination and control over its monetary policy. A few years later, in 1965, the first Cedi (¢) was introduced by President Dr Kwame Nkrumah, replacing the Ghana Pound. This change marked Ghana's move from the old imperial monetary system to a decimalised one, where one cedi was equal to ten shillings or one hundred pesewas. The term "cedi" originates from the local word "sedie", meaning cowry shell, which had been used as a traditional form of money in pre-colonial times. The introduction of the cedi represented not only modernisation but also a reclaiming of Ghana's cultural identity in its currency.

Mr Speaker, political changes soon influenced Ghana's currency system. After the overthrow of Dr Kwame Nkrumah's government in 1966, the new military regime replaced the existing cedi in 1967 with a new cedi, partly to remove symbols associated with the former president. The new currency had a conversion rate of 1 new cedi to 1.20 old cedis. While this change was politically motivated, it also demonstrated the close link between politics and monetary reforms in Ghana's history. Throughout the 1970s and 1980s, Ghana's economy experienced severe instabilities. High inflation, frequent

devaluations, and the emergence of black-market exchange rates eroded confidence in the cedi. During this period, the government implemented several exchange controls and economic reforms. The situation worsened until the Economic Recovery Programme (ERP) was launched in 1983 under the leadership of Ft Lt Jerry John Rawlings, which aimed to stabilise the economy and restore confidence in the financial system.

By the early 2000s, the cedi had suffered significant depreciation due to persistent inflation, making financial transactions cumbersome. For instance, it took around GH¢10,000 to equal one US dollar. To address this problem, the Bank of Ghana redenominated the cedi in July 2007, introducing a new currency known as the Ghana Cedi (GH¢). The conversion rate was 10,000 old cedis to one new cedi (GH¢ 1 = GH¢10,000). This redenomination simplified transactions, improved efficiency in accounting and pricing, and sought to restore public confidence in the currency.

Mr Speaker, in recent years, the cedi has continued to evolve alongside Ghana's modern financial landscape. The Bank of Ghana introduced enhanced security features and new high-value denominations (GH¢100 and GH¢200 notes) in 2019, to facilitate larger transactions. Meanwhile, the growth of digital payment systems, including mobile money, has significantly transformed how Ghanaians interact with their currency. Despite these advancements, the cedi continues to face challenges such as depreciation against

major foreign currencies, inflationary pressures, and global economic shocks.

Let all Ghanaians work towards the Ghana cedi becoming an enviable currency in the world as the journey has started from the beginning of 2025 fiscal year as the cedi is trading with other hard currencies positively. This is a good sign of a strong currency in a strong economy.

In conclusion, the history of the Ghana cedi is deeply intertwined with the nation's political and economic journey. From the colonial-era British pound to the modern Ghana cedi, each transformation has reflected Ghana's desire for stability, identity, and growth. While the cedi has faced many challenges over the decades, it remains a powerful symbol of Ghana's resilience and commitment to economic independence.

Thank you, Mr Speaker for the opportunity.

[Hear! Hear!]

Mr Second Deputy Speaker: Thank you very much. What did you say? No, not one-one. This is a very important Statement. I think, together with the Leadership, we can give opportunity for three each. Let me start with the Hon Member for Manso Nkwanta. Please, you would not contribute for more than three minutes.

Mr Tweneboa Kodua Fokuo (NPP — Manso Nkwanta): Thank you, Mr Speaker. Firstly, I commend the maker of the Statement who happens to be the

Chairman of the Committee on Economy and Development where I serve as the Deputy Ranking Member.

Mr Speaker, we congratulate ourselves as a country for having our currency for six decades. Having our own currency gives us economic independence having moved away from the British pound following independence. Having our currency, the cedi, has also given us the freedom to be able to set our own monetary policy which controls inflation, interest rates, *et cetera*, without relying on any foreign sovereign to do these for us.

Having our own currency, the cedi, for sixty years has also given us the flexibility in terms of trade, taxation and budgeting. We control our currency. There is also the integration into the global financial system. We have our currency which the world has recognised. The world trades with it. It is accepted across the world. This is a source of pride to us as a nation, and it is including us in the world platform for trading.

Mr Speaker, the cedi has shown resilience. As the maker of the Statement indicated, we have seen the notes being replaced and coins being replaced. We have seen the cedi being redenominated to make it easier to carry and transact, *et cetera*.

6:04 p.m.

The cedi has seen calm, the cedi has seen turbulence, but the cedi continues to thrive and it is our source of pride. Mr

Speaker, with these few words, I will commend the maker of the Statement and I say, long live the cedi and happy 60th anniversary.

I thank you very much.

Mr Second Deputy Speaker: Thank you very much. Let me invite the Minister for Education.

Mr Haruna Iddrisu (NDC—Tamale South): Mr Speaker, let me thank you for the opportunity to contribute to the Statement made by Hon Afful in joining the country to celebrate 60 years of the cedi. Mr Speaker, the Governor of the Bank of Ghana, at the ceremony today, under the theme, “60 years of the cedi, a symbol of sovereignty, stability and economic resilience” brought an important matter to the fore and I commend him.

The Bank of Ghana as the bankers’ bank and the lender of last resort is responsible for maintaining and monitoring financial stability and price stability. First, this is Parliament; what is the relationship between Parliament and the Bank of Ghana? Mr Speaker, we are yearning for an independent, autonomous Bank of Ghana where monetary policy is the determination of the Bank of Ghana and fiscal policy will rest in the Minister for Finance.

But Mr Speaker, in the relationship, sometimes monetary policy suffers when we do not manage our fiscal policy well. But as the maker of the Statement rightly observed, echoed by the Governor himself and our Hon Colleague who just

spoke, the cedi today is celebrated as one of the best-performing currencies in sub-Saharan Africa in, at least, the last eight months. The cedi has appreciated, Mr Speaker, according to the Governor of the Bank of Ghana, by 37.4 per cent year-on-to-date, 17th October, 2025. Again, we are told that headline inflation has dropped sharply to 9.4 per cent as of September, 2025.

Then, Mr Speaker, our observation. In 2022, the cedi depreciated by over 50 per cent, becoming the world’s worst-performing currency according to Bloomberg, and then headline inflation spiralled to 54.4 per cent. Food inflation soared to 59.7 per cent. But, Mr Speaker, this is the difficulty to manage. So, one of the fundamental problems of the John Mahama Administration is that we inherited an economy which was damaged in terms of the standing of debt. That is why we had to undergo a debt exchange programme. Apart from that, inflation was unacceptably high. But for me, Mr Speaker, as he makes that Statement, this Parliament must support the Bank of Ghana in its regulatory role. That is why they are accountable to the Parliament of Ghana. Indeed, Mr Speaker, they are expected even to give us a quarterly report. The Bank of Ghana has a responsibility to report to the Parliament of Ghana on its operations over the years.

Mr Speaker, we should also celebrate the Bank of Ghana. If for nothing, Mr Speaker, last week I was in Sunyani at the University of Energy and Natural Resources (UENR) and the Vice-Chancellor came to me and said that the

hall I was sitting in was constructed by the Bank of Ghana as part of its corporate social responsibility. I am told that if one goes to the University of Ghana and looks at the Cedi Hall, thanks to the late His Excellency Kwesi Amissah-Arthur, when he was Governor of Bank of Ghana, we have a beautiful edifice of a lecture hall which was also constructed by the Bank of Ghana. Mr Speaker, today the Bank of Ghana has a hospital which is open to the Ghanaian public.

But we want them to concentrate on their core responsibility as the bankers' bank and the regulator of the bank. We saw how Government had to spend, in the past, over GHC23 billion on what is popularly referred to in Ghana as the "collapsed Bank", or what is the name they call it? So, we pay tribute in celebrating the Governor of the Bank of Ghana, Dr Johnson Asiamah and his deputies, Dr Zakari Mumuni and Mrs Matilda Asante-Asiedu. We wish them well, but they should also recognise that others came before them and they played their part, whether it was Governor Ernest Addison or it was Governor Paul Amofo Acquah, they all did their best.

But we think that we should reposition the Bank of Ghana to take its place in modern central banking. What is the interest and policy rate? How is it determined in Ghana? How is that affecting the Ghanaian private sector and how are they able to make meaningful contribution to it? Mr Speaker, President Mahama made a promise to the Ghanaian people to establish a women's bank. Let me assure our Hon Colleagues that he would honour that promise —

[Hear! Hear!] — and a women's bank will be established as part of President Mahama's effort to empower women and to expand access to credit by them.

But, Mr Speaker, my advice is to all of us and to Ghanaians on the way we handle the Ghanaian cedi in our pockets. We love it but the way we treat it in our pockets, we do not do the same thing to the British pound or the dollar. When we get the cedi, we squeeze it as if it is some valueless paper, yet it is the paper that we expect to use for the purchase of our goods and services. So, the Bank of Ghana must also deepen public education on the way we relate to the Ghanaian cedi. Mr Speaker, we have every reason to celebrate the cedi at 60, but in celebrating it, I observe, the cedi suffered too much epilepsy in the face of the pound sterling and dollar, which reduced the value and the worth of the cedi —

Mr Second Deputy Speaker: Hon Minister, I think —

Mr Haruna Iddrisu: I do not provoke debate, Mr Speaker. So, I want to commend the maker of the Statement, congratulate and commend the leadership of the Bank of Ghana, past governors of the Bank of Ghana, past chairs of the Bank of Ghana, and then to Parliament. Mr Speaker, there is no day I will comment on Bank of Ghana without recognising the late Anthony Akoto Osei, of blessed memory, when we had to review the Bank of Ghana Act, 2002 (Act 612) as to whether the Bank of Ghana can finance Government or not. Part of the crisis we are in as a country,

what we witnessed in 2022, was the Bank of Ghana financing Government. That is what resulted in now, the debt exchange and the slippage of the economy. The late Hon Anthony Akoto Osei sat here every other time to caution the Bank of Ghana, and he was a strong advocate of zero financing of Government by the Bank of Ghana. I pray that we will keep faith to it.

Now, gold reserves, Mr Speaker. Gold reserves are everywhere in the world. We should learn from best practice and allow for efficiency in improving the standing of our gold reserves. So, I commend Hon Afful and I thank him for the Statement.

Mr Speaker, thank you for tolerating —*[Hear! Hear!]*

Mr Second Deputy Speaker: Thank you very much. So, Leadership — But I said three, including Leadership. All right, Dr Mahama?

Dr Abdul Kabiru Tiah Mahama (NPP — Walewale): Mr Speaker, thank you very much. Let me join my Hon Colleagues in commending Hon Eric Afful for making this Statement on the 60th Anniversary of the Ghanaian cedi. Mr Speaker, I do not need to go back to the history of the Ghanaian cedi, as there are many grey-hairs here who understand this better than I do, and the maker of the Statement also attempts to capture that particular history.

Mr Speaker, the fact is that after 60 years, the Ghanaian cedi has not found its feet. I would think that in this moment

of reflection, we need to ask ourselves why after 60 years our cedi is still trying to find its feet against the major trading currency. We understand our currency is not a vehicle currency, but we do also appreciate that for an economy to be very strong, we need to have our cedi, medium of exchange or currency to be very stable for predictability of business.

6:14 p.m.

We need to have it very stable for the health of the economy and even for the purchasing power of the ordinary person. So, we all agree and admit that we must do something strong about the performance of our cedi.

Mr Speaker, as we celebrate 60 years, we have to pay some glowing tribute to other individuals. He mentioned some names, and I think it is important that we cannot celebrate 60 years of independence and the history of the currency is being talked about without reference to Dr Mahamudu Bawumia. When H.E. Dr Mahamudu Bawumia was at the Bank of Ghana, he was the arrowhead of the re-denomination exercise that brought some stability to the Ghanaian cedi. This is a statement of fact, and we need to pay a glowing tribute to him.

Mr Speaker, my senior Brother, the respected former Leader of the House and Minister for Education drew our attention to the need for us to support our currency with commodities such as gold, and that the African Development Bank (ADB) and international communities are now coming to the realisation that we

need to support our currency with a very predictable commodity such as gold.

When the former Vice President came up with this idea, people said it was not novel; people were mocking at him. Mr Speaker, the chicken has come home to roost. I am happy that the Governor of the Bank of Ghana can now go on international platforms to count the success of the country on the back of gold reserve programme and gold purchase programme. I am happy that we are building a country and the contribution of Dr Mahamudu Bawumia needs to be highlighted.

Mr Speaker, what can we do? Currently, the performance of the cedi is because it is being given an aphrodisiac, and if the cedi is performing because it is given an aphrodisiac, you do not call that natural performance. Mr Speaker, we need our cedi to perform naturally. How do we do that? We need to improve on the real sector of the economy. That is the only way. Otherwise, all the performance we are seeing about the cedi is not based on productivity.

We are learning that, this month alone, the Bank of Ghana is pumping over 1.3 billion to support the cedi. This is not economic management; this is intensive care management of the cedi, and I do not think that that is the way we need to go with the management of the cedi.

Mr Speaker, as we celebrate 60 years, we want to also advise that there is the need for us to streamline the black-market rates with the formal rates. I do

not understand why *abokyi* will be charging us GH¢13.00 to the dollar, and the Bank of Ghana is quoting GH¢11.00 to the dollar. It does not make sense. There must be some symmetry. There must be some consistency. The differences should not be that—

Mr Speaker, I will conclude on this note. The maker of the Statement is referring to some exchange rate that was GH¢17.00 to the dollar. I will come on the ruling of this particular House. The House has ruled on January 26, 2017, and it is based on evidence to support a statement of fact. The issue at the time was whether or not a Member can continue to speak when challenged on a statement of fact. Mr Speaker, there is a statement of fact that has been mentioned that the cedi was changing to the dollar for GH¢17.00. I think based on the ruling of this House, we have to expunge that from the Statement. Because the ruling of this House is clear on such matter that where a Member of this House is challenged on a purported statement of fact, the Member must support the statement with evidence or withdraw the unsubstantiated statement.

Mr Speaker, this was the ruling of the House on the 26th January, 2017. So, I want to—

Mr Second Deputy Speaker: 2017?

Dr Mahama: Mr Speaker, I want to urge my Colleague that this part of the Statement should be expunged from the record, or he provides the evidence to substantiate that. Since he has not provided evidence on the Statement,

prima facie, that Statement should not be part of the records of this House. With these few words, Mr Speaker, I thank you very much. [*Hear! Hear!*]

Mr Second Deputy Speaker: Very well.

Let me give the Deputy Minister the opportunity.

Ms Rita Naa Odoley Sowah (NDC — Dadekotopon): Thank you, Mr Speaker, for the opportunity. I want to start by congratulating the whole of Ghana for the celebration of the Cedi at 60. The cedi which came into being in 1965 is 60 years now and it is going to be a year-long celebration. Believe you me, it has gone through various reforms and we are still using the cedi.

As we speak now, in the tenure of His Excellency John Dramani Mahama, we all see how the cedi is performing against the dollar. We recall in 2024 where the cedi was performing very badly against the dollar. So, you will see clearly that this is an administration that wants Ghanaians to see the value of the cedi as we celebrate the Cedi at 60. Mr Speaker, I would want to join the Bank of Ghana, the Governor and his team for this year-long celebration and I believe that the cedi would perform better even as the days and years roll by.

Congratulations to Ghana,
congratulations to all of us.

Mr Second Deputy Speaker: Hon Oppong Nkrumah?

Mr Kojo Oppong Nkrumah (NPP — Ofoase Ayirebi): Mr Speaker, thank you for the opportunity to contribute to this Statement. I want to commend the Chairman of the Committee on Economy and Development for this commemorative Statement. Mr Speaker, as we celebrate 60 years of the Ghana cedi, to many Ghanaians, the most important question is what is the value of the cedi? What can it really buy? That brings to mind two specific questions; the question of inflation targeting and inflation management, and exchange rate.

Mr Speaker, if you study the economic history of our country, our cedi performs very well at some point in time and also very poorly at some point in time. Whenever a government has access to large volumes of forex reserves to do what they call market intervention, the cedi is able to stand its ground against international currencies. Whenever we run short of hard currency, the depreciation gets very dire.

Today, I am happy to reiterate the comments made by the Minister for Education that the Governor announces about a 37.4 per cent appreciation of the cedi year to date. The question is how? I think the Government has answered it already by explaining that it is because they are doing market intervention. In fact, this month alone, as was mentioned by one of my Colleagues, the Bank of Ghana is dumping about 1.3 billion dollars onto the market. That is a reason for the appreciation. So, where are they getting these dollars from?

Mr Speaker, it is because in 2021 a gold purchase programme was introduced for the first time in our history, aimed at enabling the state to, through the Bank of Ghana and the Precious Minerals Marketing Company (PMMC), buy gold and increase our gold reserves. That is how we can move from 8 tonnes to 30 tonnes.

Mr Speaker, at that time, gold prices were averaging about US\$1,700 per ounce. Today, gold prices are averaging about US\$4,000 per ounce. So, because of this gold purchase programme, we are able to mobilise a lot of forex and the central bank is now doing a lot of market intervention with the forex. Has anything strange happened? No. It is because of the conditions of the market and the introduction of the gold purchase programme that today we are able to mobilise a lot of forex through the Bank of Ghana and the GoldBod, which used to be the PMMC, and this forex is now being dumped on the market.

So Mr Speaker, it begs the question that if you leave the market to the normal forces of demand and supply for currency without a market intervention, would we be getting a 37.4 per cent appreciation? Respectfully, I doubt. That then raises the second question of what framework is being used today that we are getting windfall? Are we dumping all the windfall on the market to suppress the real rate? It would be recalled that earlier when the Governor and the President denied it and the International Monetary Fund (IMF) cautioned the government to pause and put in place a framework and they did, the currency

started depreciating again. And indeed, the President at the press conference admitted that it was because the Bank of Ghana had stopped the market intervention.

6:24 p.m.

So, Mr Speaker, we do not need to split heads about it. Today, as we commemorate 60 years of the cedi, we need to be honest with ourselves that, it is the market intervention and any government that is able to benefit from these prices on the market and has these reserves, will intervene. It is like somebody with a blood pressure (BP) problem who is taking blood pressure medication to bring his BP down, now claiming all of a sudden that his BP is sorted. The day the person stops taking the medication, that is when the person will see their smoothness level.

Mr Speaker, what we need to do is to have a discussion about the sustainability. How are we going to ensure that on a sustainable basis? We can allow the market forces to hold our currency where it has to be without the aphrodisiac and the blood pressure medication that my Colleagues and I talked about. That requires the Bank of Ghana to do a lot more of the traditional things, ensuring that forex earnings are repatriated into the jurisdiction, ensuring that the banks do not hold the forex earnings in their nostro accounts, ensuring that the de-dollarisation, as the Chairman mentioned, really comes to bear. These things are not being done. It is the market intervention. So, today, as we commemorate and celebrate, my

prayer is that we pay more attention to the sustainable things.

The last thing, Mr Speaker, is that a proper framework needs to be put in place. Is it 10, 15, 20 or 100 per cent of the windfall that we are dumping on the market? And if we dump all of it, what will happen when it is finished? We commend the various actors from 1965 who have worked to bring the Ghana cedi to where it is and we pray the current administration to put in place a sustainable framework so that when gold prices are no longer at US\$4,000 on the world market, we do not see a further depreciation of the Ghana cedi.

Mr Speaker, I thank for the opportunity. *[Hear! Hear!]*

Mr Second Deputy Speaker: Thank you very much. I will take the last one from Hon Shaibu.

Mr Shaibu Mahama (NDC — Daboya/Mankarigu): Mr Speaker, let me thank you very much for the opportunity and to thank the maker of the Statement for teasing out a very critical issue regarding our economy and our cedi.

Mr Speaker, 60 years in the life of a living being, and indeed a cedi, is so important that we need to celebrate it. Indeed, the Ghana cedi has travelled so many years. It has transitioned from one point to the other and today, we have a Ghana cedi that is stabilised.

Mr Speaker, my good Friend made mention of aphrodisiacs. Let me be clear

to point out that there are good aphrodisiacs and bad ones and depending on the time that one injects the aphrodisiac, one will get a result that one wants. Indeed, some years ago, some of the aphrodisiacs were pumped into the system and we got GH¢17 to US\$1. Indeed, today, a better aphrodisiac is pumped into the system and we are hitting between GH¢10 to GH¢12 to US\$1. This is proper and prudent management.

My learned Friend, Hon Kojo Oppong Nkrumah, mentioned good management. Indeed, policies were put in place and at the time those policies were put in place, we still had the cedi falling to GH¢17.

Today, we have put in a very prudent measure. We have established the Gold Board. The 1.3 billion that was pumped into the system was not borrowed externally but was mobilised locally. Indeed, from January to October 15, the Gold Board alone has raked in over US\$8 billion and that is sitting in the coffers of the Bank of Ghana.

Mr Speaker, today the cedi shines very brightly and the clients I represent tell me to take their rent in Ghana cedi. They ask me not go for any dollar for them, even though it is said the dollar denominates. People now prefer to use the Ghana cedi because of the value it presents. That is the cedi we are talking about today and that is a cedi we are celebrating.

So, the prudent economic measures that have been put in place is the reason

we are all smiling today. And then take note that the Ghana cedi will continue on an upward surge. And this economy and the Bank of Ghana is up to recovering. Please let us wish it very well. We wish the Ghana cedi a happy birthday and we pray that we all hold it and keep it well. I thank you so much, Mr Speaker. *[Hear! Hear!]*

Mr Second Deputy Speaker: Thank you. Leadership?

Yes, Leader.

Mr Jerry Ahmed Shaib (NPP — Weija-Gbawe): Mr Speaker, thank you very much for this opportunity and I want to commend the maker of this Statement, save that, when we get an opportunity to make such Statements, we should try and de-politicise the content of the Statements and see the contributions and evolution the cedi has gone through to this stage. It is very critical that as Ghanaians, we all come together to celebrate the cedi at 60. This is not just by chance; it is by hard work and the very critical support of many stakeholders.

Mr Speaker, I always take a cue from my former Leader and Minister of Education and therefore, I want to look at it from the angle of not just the evolution of the cedi, but the use of the cedi. For someone to have a Statement of this nature and even talk about a cedi as being GH¢17 as at 2023 and 2024 is not factual. We cannot be in the House of facts and records and state misfacts. It is unfortunate for anybody to indicate that, especially when there is no evidence to

that effect. The cedi has never gone to GH¢17. We have done propaganda with the cedi. We have done propaganda at all times material. When the cedi was even GH¢14, we were pushing it to GH¢17. Like we are talking today, the cedi is on an aphrodisiac. My Ranking Member on the Committee on Assurances can tell you the kind of aphrodisiac the cedi is on and the Hon Member should also be mindful that not all aphrodisiacs are effective. *[Laughter]* Some aphrodisiacs can actually dismember someone. It can give a heart attack. When they even put oxygen on the person, it will not survive. One of the aphrodisiacs is propaganda. The other aphrodisiac is non-payment of contractors. Pay contractors and let us see that the cedi is standing on its knees. All we come and do here is the usual propaganda. The cedi is 60 and the Government has been able to—Are we not Ghanaians? If the cedi is stable and solid, it is to the benefit of all the Ghanaian people, including us. When we do not know how Gold Board is funded, and we do not know what is happening to it, they do not want us to know how the cedi is performing. Let us know?

When there is constant interference from the Bank of Ghana, no, let us talk about the realities on the ground. People have done well, let us commend them. I want to look at it from the angle that whoever has contributed to the proper evolution of the cedi must be commended and celebrated. I think we have a list of names. Yes, President Mahama is one of them. Anybody who has been a president since the inception of the cedi should be commended. Nana Addo Dankwa Akufo-Addo who has

been the President of the Republic of Ghana for eight years is a major contributor of the stability of the cedi. And when we are talking, the Hon Member for Odododiodio is saying that H.E Akufo-Addo has not done anything. I need the Hon Member to understand that President Kufuor, Dr Mahamudu Bawumia, all the Governors, Deputy Governors—

6:34 p.m.

Mr Speaker, I travelled to Barbados recently and I had to use cash. I have always held on to cash because I believe that sometimes these systems can break down. I was just telling my boss that I used American Airlines and to just pay for an extra bag, I was asked to use a card. Mr Speaker, I do not have a card, I still do not. I had to look for someone in the queue who could be of help. So, somebody paid and I gave the person cash. All of this has been part of the evolution of the cedi. It does not have to always be cash; it can be through card.

We started the digitalisation of the cedi, Mr Speaker. And today, you know what is happening to the Lightwave Health Management System (LHIMS)? The LHIMS is not working because we are back to cash. *[Uproar]* **[An Hon Member:** You cannot just be shouting; sit down] Mr Speaker, the fact that we are back to cash alone tells you that we have done well as a nation but we are departing. For the fact that people go to the hospitals and they cannot access information, just because of the cashless system—

Mr Speaker, it is very critical to cherish the capacities of the people who have channeled the cash into a cashless system, and we want to commend the Bank of Ghana for doing the needful and for celebrating the 60 years. We are hoping that all Members of Parliament would actually have benefited from maybe a memory form of cash from the Bank of Ghana. So that when we go home, we can also show that we were part of the 60th anniversary celebration of the cedi.

Thank you very much, Mr Speaker.

Mr Second Deputy Speaker: Thank you very much. Yes, Majority Leadership? Hon Member for Odododiodio, I have begged you.

Ms Comfort Doyoe Cudjoe: Thank you, Mr Speaker, for the opportunity to add my voice to the Statement made by my Colleague, Mr Eric Afful.

Mr Speaker, the cedi is 60 years old and we appreciate where it is coming from and where it has reached now. How do we see our cedi? Let us all look at how we see the cedi. The cedi should be respected when it gets into our hand. We must see it as something that is valuable. We should not be squeezing it. When we get a dollar bill, we quickly look for a wallet to keep the bill to preserve it. But when we get cedi, then you see people looking for cloth to tie it in as if the cedi is nothing to write home about.

Mr Speaker, let us give the respect we give to the dollar to the cedi. How we see our cedi is how anybody else too sees

it. When the cedi is dirty, the Bank of Ghana should please change it. Let us have clean money so that anywhere anybody sees our cedi, the person will give the cedi the respect that it deserves.

Currently, the cedi is doing well. The whole world is commending Ghana because the cedi is doing well. Mr Speaker, we have the men and the men are working on the cedi and it is really performing. *[Hear! Hear!]* If your mother is dead and you say your mother is sleeping, when mothers are feeding their children in the evening, nobody will come out to feed you. Mr Speaker, so if the dollar appreciated against the cedi so that we had a dollar to GH¢17, and they said it is not true, Ghanaians will judge. Now the cedi is doing well against the dollar globally. So, we should applaud the Government in power for holding the cedi strong for all of us.

Mr Speaker, respect for the cedi is what I am calling for. Let us go to the marketplaces and educate the market women on how to handle the cedi. How you sell yourself is how people will buy you. How do we see the cedi? Recently when we went to the IPU, I did not see myself as an elderly person. I saw myself as a young person and I fought for a position and won. *[Hear! Hear!]* So let us see the cedi as valuable, and the cedi

will be respected globally as it is being recognised now.

So, Mr Speaker, I congratulate the Governor and all the past Governors from 1965 to now. Everybody has done their part and we will continue to do ours to preserve the cedi for the next generation.

Thank you, Mr Speaker.

Mr Second Deputy Speaker: Thank you very much.

Hon Members, we will now bring comments on the Statement to a close. We have done almost all the Business on the *Order Paper*. I will revert to Leadership. It is not yet 7 o'clock so you would probably have to move a Motion for adjournment.

Ms Comfort Doyoe Cudjoe: Mr Speaker, I beg to move that this House be adjourned till tomorrow at 2 o'clock p.m.

Mr Jerry Ahmed Shaib: Mr Speaker, I beg to second the Motion.

Question put and Motion agreed to.

ADJOURNMENT

The House was accordingly adjourned at 6:42 p.m. till Wednesday, 29th October, 2025, at 2:00 p.m.

